Confidentiality and Privileged Communication

5

LEARNING OBJECTIVES

5.1 Demonstrate an understanding of the difference between confidentiality and privileged communication.

5.2 Explain how concerns about client confidentiality have changed with the increase in virtual counseling that has taken place since the beginning of the COVID-19 pandemic.

5.3 Identify some common exceptions to confidentiality in counseling sessions.

5.4 Explain legal principles regarding the rights of parents or guardians when counseling minors.

Confidentiality is one of the most fundamental of all professional obligations in counseling. It is also one of the most complex and problematic duties to manage in day-to-day practice. Consider, for instance, what you would do if you were the counselor in each of the following situations.

5-1 The Case of Elena

Elena has been working with Pete, aged 28, who came for counseling to resolve some family-of-origin issues that he believes are creating problems in his relationship with his partner, Jacob. During their last counseling session together, Pete tearfully revealed that he recently discovered that Jacob is having a sexual relationship with a mutual friend of theirs named Alex. Pete has read email messages between them that clearly indicate they are having an affair. Jacob and Alex do not know that Pete knows about their affair. Pete told Elena that he is very angry with both Jacob and Alex and recently has had fantasies about killing both of them because they have betrayed him.

FOCUS QUESTIONS

1. What do you think are the distinctions among the terms *privacy, confidentiality*, and *privileged communication*?
2. Some studies have shown that clients are not very concerned about privacy or confidentiality when they seek counseling services. What do you think about such findings?
3. How would you respond if a client asked to see the notes you have taken related to their counseling sessions?
4. What do you think should happen to clients' records after they die?

- How should Elena determine whether Pete is serious about possibly killing Jacob and Alex?
- As Pete's counselor, is Elena obligated to warn Jacob or Alex about Pete's statement that he has had fantasies about killing them?

Discussion: Counselors have both an ethical and a legal (except in Texas) duty to take action to prevent harm if they believe their client is serious about planning to harm another specific person. The legal duty exists (except in Texas) because of the *Tarasoff* case (*Tarasoff v. Regents of University of California*, 1976), which is discussed in detail in Chapter 8. It is important to realize that sometimes clients make statements about killing themselves or others but they are simply upset and overstate their distress.

In this situation, Elena must ask Pete specific questions to determine whether he might be serious about possibly trying to kill Jacob or Alex or both of them. Elena has to do an assessment to determine whether Pete might be at risk of seriously harming Jacob or Alex. In most cases of this kind, after asking specific questions, Elena probably will determine that Pete is upset but has no intentions of killing either Jacob or Alex. If she decides that Pete is not at risk of seriously harming Jacob or Alex, then Elena should ask Pete to let her know if he feels differently in the future and should enter into her notes what Pete said, the questions she asked Pete, his answers, and her conclusion that he is not currently at risk of seriously harming anyone.

If, after completing a risk assessment by asking Pete a series of questions, Elena determines that Pete might pose a risk of seriously harming Jacob or Alex, then she will have to take action to prevent the harm. The action Elena must take varies from state to state and depends on state laws that might have been enacted related to the duty to warn intended victims. If Elena determines that Pete may be at risk of harming others, the best result would be to convince Pete to submit to an evaluation at a hospital so that he could be committed if necessary. If Pete would not agree to voluntarily submit to an evaluation in a hospital, then Elena will need to report her concern to legal authorities, and in some states, warn the intended victims. Other options for Elena in such dire situations are discussed in Chapter 8.

5-2 The Case of Nancy

Nancy, a counselor in private practice, has been conducting a group on Zoom for adolescent girls aged 14–16. The parents of the girls brought their daughters to Nancy for counseling and agreed that their daughters could participate in the group counseling process. The focus of the group is on building self-esteem and making wise choices. During the fourth session today, the group members begin discussing boyfriends and whether to *just say no* to sex. Marlee, a 15-year-old freshman, shares that she and her boyfriend are having sexual relations and that she thinks it's okay because they are really in love. She mentions, almost in passing, that her boyfriend is 20 years old and that her parents would *kill her* if they found out about him.

- Can Marlee's counselor keep her disclosure in confidence, or must she notify Marlee's parents?
- If Nancy breaches confidentiality, what are the risks to her relationship with Marlee?

- If Nancy breaches confidentiality, what are the risks to the group process, to the trust that other group members may have come to feel toward her, and to her reputation for trustworthiness among the student population in general?
- Are these risks greater than the risks posed by keeping Marlee's relationship with her boyfriend a secret from her parents?

Discussion: Nancy cannot keep Marlee's disclosure a secret; she must notify Marlee's parents, although she should first inform Marlee of her obligation and explain why she cannot keep the secret. If Nancy does not tell the parents and Marlee becomes pregnant or is harmed by the relationship with an adult male, the parents might attempt to hold Nancy responsible. Nancy should check her state law, as she may also be required to report suspected child abuse to authorities. Whether Nancy has to report suspected child abuse depends on the language in the statute in her state.

It certainly is possible that Marlee will be very upset after Nancy tells her parents, and Marlee may not want to continue the counseling relationship. Nancy should do her best to maintain the relationship. This may be possible if she is able to convince Marlee that it is in her best interest for her parents to know, or if she can obtain Marlee's agreement that Marlee will tell her parents, perhaps in Nancy's office with Nancy present.

This group might not continue, although Nancy should try to keep it functioning and work through the issue with the other students. Hopefully, Nancy has already discussed the limits of confidentiality with the group members so they will understand why she cannot keep Marlee's secret. It is likely, however, that Nancy's reputation among adolescent clients for trustworthiness will be compromised, and it is possible that the other group members will tell their friends that Nancy will tell their parents what they say if they go to her for counseling.

After pondering these scenarios, you might not be surprised to learn that counselors encounter dilemmas of confidentiality more frequently than other types of ethical challenges and find them the most difficult to resolve (Fisher, 2008). Questions surrounding confidentiality can be very complex and often involve legal as well as ethical considerations. Fisher (2016) has developed an e-book that leads mental health professionals through the decision-making process for determining when to disclose confidential information.

A useful starting place may be to clarify the distinctions among three terms—*privacy, confidentiality*, and *privileged communication*. These terms are sometimes used interchangeably by counselors, but the concepts have important differences. Both confidentiality and privileged communication arise from our societal belief that individuals have a right to privacy. *Privacy* is the broadest of the three concepts and refers to the right of persons to decide what information about themselves will be shared with or withheld from others. Confidentiality and privileged communication both apply more specifically to the professional relationship between counselors and clients. *Confidentiality* is primarily an ethical concept that refers to the counselor's obligation to respect the client's privacy and the promise to clients that the information they reveal during counseling will be protected from disclosure without their consent. *Privileged communication* is the narrowest of the three terms and is a legal concept. Privileged communication laws protect clients from having confidential communications with their counselors disclosed in a court of law without their permission (Garner, 2014). For a communication to be privileged, a statute must have been enacted that grants privilege to a category of professionals and to those whom they serve.

The Impact of the COVID-19 Pandemic on Confidentiality and Privileged Communication

It will be some time before ethics codes, statutes, and case law are created or revised to address the issues of privacy, confidentiality, and privileged communication in the current digital environment. The COVID-19 pandemic arrived unexpectedly and counselors, supervisors, and counselor educators are doing their best to ensure the privacy of clients is protected in the rapidly evolving world of virtual counseling.

Despite concerns about telecommunications sometimes being hacked or compromised since the COVID-19 pandemic began, counselors generally have found that clients feel secure using encrypted HIPAA-compliant telecommunications for video meetings and communicating by text, email, and telephone calls. Even before the pandemic, paper or recorded counseling records were vulnerable to exposure. The same is true for digital communications and recordings. Counselors have an obligation to take reasonable steps to ensure digital communications, recordings, and records related to their work with clients are secure. Today, reasonable steps include ensuring that secure telecommunications are utilized, that third parties do not hear counseling sessions as they occur, and that electronic records are stored appropriately. Maxim et al. (2022), Rollins et al. (2022), and Wootton et al. (2020) have provided guidelines for ensuring that privacy is maintained using Zoom and other digital platforms when conducting counseling sessions and supervising counselors in training. To ensure client privacy, counselors should use encrypted video platforms; ensure no one else can hear the counselor and client when counseling is taking place; ensure any recordings are stored in a password-protected location; and warn clients that they would be risking their privacy if they are not in a private place when counseling is taking place. An essential part of preventing privacy problems when providing virtual counseling is to be properly trained in using the platform of choice and to ensure technical issues do not interfere with the quality of sessions.

In this chapter, we first explore confidentiality as an ethical issue and then discuss privileged communication. Later, we look at the numerous exceptions that exist, both to confidentiality and to privilege.

CONFIDENTIALITY

Origins of Confidentiality

Helping professionals assume that an assurance of confidentiality is an indispensable requirement for effective counseling and that, without this assurance, many clients would not feel safe to discuss openly the intimate and personal aspects of their lives or would not seek counseling (Mosher & Berman, 2015). Actual studies, however, show only mixed support for this assumption. Some studies have supported the belief that privacy assurances are essential (Alderman, 2017; Leichliter et al., 2017; Whetten-Goldstein et al., 2001), whereas other studies have indicated that such assurances have little effect on encouraging disclosures (Dienlin & Trepte, 2015; Peter & Valkenburg, 2011; Steijn, 2014; Steijn et al., 2016), or that limits to confidentiality matter only to some clients in some circumstances. Despite this lack of unequivocal evidence, confidentiality has become such an internalized norm in the counseling profession that it is rarely questioned. A brief look at some of the historical origins of the profession may be helpful in understanding how this norm developed.

Counseling represents the fusion of many diverse influences. One of these influences was the emergence of the field of psychiatry in the treatment of mental illness. Almost until the beginning of the nineteenth century, mental illness was viewed as mystical and demonic. These early times

are associated with images of "lunatics" in chains in asylums. It was not until the 1800s that significant strides were made in the understanding of mental illness, and not until the 1960s that the deinstitutionalization of the mentally ill brought these individuals back into contact with society. Another force that bears mentioning is the emergence of psychoanalysis in the early to middle twentieth century. Patients of Freudian analysts were expected to work through their socially unacceptable urges, sexual fantasies, and repressed feelings and thoughts, and to do so in a society that held Victorian social mores. Early notions about mental illness and negative impressions about the nature of personal material discussed in analysis helped to create a social stigma. In this climate, it is understandable that the patient's need for absolute privacy was assumed and that people would want to hide any information related to their having sought and received treatment.

It was not until the middle of the twentieth century that other, countervailing influences emerged in the mental health field. Carl Rogers's humanistic views, other theorists who emphasized the natural developmental life stages that all individuals pass through, and the career guidance movement all helped to shift thinking away from counseling as a service only for the mentally ill or the sexually repressed. Concurrently, as scientists began to discover the biological bases for some mental disorders and to find medications that could alleviate conditions formerly thought to be untreatable, the stigmatization associated with mental illness and psychotherapy began to decrease. Nonetheless, even in current society, a notion stubbornly persists that there is something shameful about seeking the services of a mental health professional. Unfortunately, many members of today's society would likely not question the language used by the U.S. Supreme Court in its 1996 decision in *Jaffee v. Redmond et al.*, p. 8, a case we describe later in this chapter:

Because of the sensitive nature of the problems for which individuals consult mental health professionals, disclosure of confidential communications made during counseling sessions may cause *embarrassment or disgrace*.

The Rationale for Confidentiality

Whatever its origin, confidentiality is universally viewed today as being essential to the counseling process, which depends on an atmosphere of confidence and trust in which clients are able to tell their stories freely and to honestly disclose their feelings, fears, thoughts, memories, and desires. Clients need to know that they can trust their counselors to respect their privacy, and the counselor's confidentiality pledge is the cornerstone on which this trust is built (ACA, 2014, §A: Introduction).

Confidentiality is a strong moral force that helps shape the manner in which counselors relate to their clients. Bok (1983) has suggested that confidentiality is based on four premises. The first two premises relate to respect for client rights in counseling. The principle of *respect for autonomy* (which is described in Chapter 1) means that counselors honor their clients' ability to be self-determining and to make their own choices. Applied to confidentiality, it also means that counselors honor the rights of clients to decide who knows what information about them and in what circumstances. The second premise is *respect* for human relationships and for the secrets that certain types of relationships entail. In the professional relationship, the intimate context in which these secrets are shared is seen as essential to counseling. The third premise has to do with the counselor's obligation that arises from autonomy and respect. Bok contends that an additional duty is created by a *pledge of silence*, which is the offer of confidentiality extended by the counselor to the client. Counselors are bound to this pledge both in word and in deed; when they have given their word, they are obligated to actively work to protect clients' secrets from disclosure. The final basis for confidentiality is its *utility*. The rationale here is that confidentiality in counseling relationships is useful to society because clients would be reluctant to seek help without an assurance of

privacy. Society gives up its right to certain information and accepts the risks of not knowing about some problems and dangers in society in exchange for the benefit that is gained when its members improve their mental health.

Respect for autonomy is only one of the moral principles on which confidentiality rests. Another is *fidelity*, which means being faithful and keeping promises, as we describe in Chapter 1. Certainly, one of the most important promises that counselors make to clients is that they will keep the secrets shared in counseling. Today, confidentiality is an issue not only of counselor belief systems but also of consumer rights. U.S. culture has placed increasing emphasis on the rights of service recipients, and clients are more likely to hold an expectation that their privacy will be maintained by the professionals whose help they seek. This expectation of privacy has important legal as well as ethical implications, as you will discover later in this chapter.

Counselor Practices and Confidentiality

How well do practicing counselors deal with confidentiality issues? Some evidence suggests that counselors feel confident of their ability to make sound ethical judgments about confidentiality issues. Probably because counselors have developed a good understanding of their obligations around client confidentiality, very few refereed journal articles on the topic have been published in recent decades. Gibson and Pope (1993) surveyed a large national sample of counselors regarding their beliefs about whether a wide range of behaviors were ethical or unethical and how confident they were in making these judgments. Twenty-nine percent of the items for which the confidence rating was very high (at least 9.0 on a 10-point scale) concerned confidentiality. In a more recent survey, Neukrug and Milliken (2011) asked American Counseling Association (ACA) members to rate 77 behaviors as ethical or unethical. In their study, two confidentiality-related behaviors about which the respondents were in overwhelming agreement (over 95%) were that it is ethical to break confidentiality if a client is threatening harm to self and that it is unethical to fail to reveal the limits of confidentiality to a client.

Counselors' confidence in their ethical judgments may be well founded. Complaints made against helping professionals for violations of confidentiality are not as common as one might expect. Various studies have shown that only 1% to 5% of complaints made to the ethics committees and state licensing boards of counselors involve violations of confidentiality (Anderson & Freeman, 2006; Garcia et al., 1994; Garcia et al., 1995; Neukrug et al., 1992). It appears that mental health professionals take seriously their pledge to maintain their clients' confidentiality and are diligent in honoring it. Grabois (1997/1998) reported that there have been only a few cases in which mental health professionals had been sued for breaching confidentiality. Counselors should not become complacent about their confidentiality practices, however. Statistics regarding formal complaints may not accurately reflect the actual frequency of breaches of confidentiality. According to one study, 61.9% of psychologists reported that they had *unintentionally* violated their clients' confidentiality (Pope et al., 1987). Clients may not have been aware of these unintentional breaches. Although it may be startling to think that the majority of practitioners may have violated their clients' confidentiality, it is somewhat understandable when we pause to consider the myriad ways that an inadvertent or careless breach could occur. The following examples illustrate this point.

> Counselors do not always have the luxury of having a separate entrance and exit for their clients to use. When clients who are leaving their sessions must pass through a waiting area, counselors need to take special precautions to guard their clients' privacy. We know one very conscientious counselor in private practice who routinely left 15-minute intervals between the end of one scheduled appointment and the beginning of the next and who avoided scheduling consecutively any clients from the same part

of town. Nevertheless, one day she ran late with a client, who exited into the waiting room and ran into a friend from church. Both clients appeared uncomfortable to have encountered each other in this way.

Counselors are encouraged to consult with each other regarding clients who present challenges to them, and most consultations can be managed without revealing a client's identity. However, it is important not to obtain a colleague's advice while walking through the halls of an agency or institution, at a restaurant, or in another public place. Pope and Vasquez (2016) related the story of a counselor who was consulting a colleague about a particularly *difficult* client while they were on a crowded elevator. The counselor did not mention the client's name but gave enough detail that the client, who happened to be standing only a few feet behind them in the elevator, was able to ascertain that they were the subject of the discussion and to listen with intense interest and dismay.

In our environment today, much counseling takes place virtually, creating even more ways that counselors could unintentionally compromise a client's privacy or confidentiality. For example, a counselor's interactions with a client might be overheard by a family member or someone else without the counselor realizing it; a recording of a client session might be mistakenly transferred to a computer folder that has open access, or a private email or text to a client might be accidently sent to someone else.

Clearly, it is easy for even seasoned practitioners to violate a client's privacy unintentionally. Students in counselor training programs must also take special care to maintain client confidentiality. Although it may be tempting to share with family, friends, or fellow students your excitement about what you are learning in a practicum, a practice session with a peer client, or an experiential group, it is important to keep in mind that the ethical obligations of students are the same as those of practicing counselors (ACA, 2014, §F.5.a.). Your conversations must not reveal any information that could conceivably allow listeners to ascertain the identity of a client. Case notes, test protocols and interpretations, audiotapes, and videotapes of work with clients must not be left in places where they might be seen by anyone other than authorized supervisors or professors. It is a good idea to get into the habit of zealously safeguarding client confidentiality from the very beginning of your graduate studies.

A question that surfaces quite often for beginning counselors is whether they have an obligation to maintain the confidentiality of private information that is disclosed to them when they are not in the role of counselor—for example, when a neighbor tells a counselor about an event that occurred a few houses away. The answer is that confidentiality and privileged communication exist only when counselors are functioning in the role of a professional counselor. As explained by Sydow (2006), courts have held that—even when a counselor is at work—privilege exists only when a counselor–client relationship exists and when the counselor is providing professional counseling services.

Ethical Standards and Confidentiality

Confidentiality is the only specific ethical issue to which an entire section of the *Code of Ethics* (ACA, 2014) is devoted. Section B: Confidentiality and Privacy emphasizes the client's right to privacy in the counseling relationship. Exceptions and limitations to confidentiality are addressed, and because exceptions to confidentiality may be mandated by legal as well as ethical considerations, the standards are carefully written to minimize conflicts between law and ethics. This section addresses special circumstances in counseling minors, families, and groups and offers guidelines for maintaining confidentiality when consulting or conducting research.

The Introduction to Section B is general and states, "Counselors recognize that trust is a cornerstone of the counseling relationship." An early standard in this section states, "Counselors

respect the privacy of prospective and current clients" (§B.1.b.). Sometimes counselors, in their zeal to protect a client's privacy, mistakenly believe that they should maintain a client's confidentiality even when the client asks them to share information with others (Herlihy & Corey, 2015a). When clients request that information be disclosed, counselors should honor these requests. If the counselor believes that releasing the information might be detrimental to the client's best interests, these concerns should be discussed with the client, but the ultimate decision belongs to the client. In many instances, client requests for disclosure are not particularly problematic for counselors, such as when clients move and later request that their records be sent to their new counselor. Other situations may cause counselors some discomfort, for example, when clients ask to see their own records. It is important to realize that records are kept for the benefit of clients and that counselors are obligated to provide clients with access to their records, unless the records contain information that may be misleading or harmful to the client (ACA, 2014, §B.6.e.). The Health Insurance Portability and Accountability Act (HIPAA), a federal statute that is discussed at length in Chapter 6, requires that clients be given access to their counseling records except in exceptional circumstances in which clients may be harmed by seeing their records.

One of the reasons that confidentiality is such a difficult ethical issue is that *confidentiality is not absolute.* Although counselors should make every effort to prevent inadvertent breaches, sometimes confidentiality *must* be breached. Counselors must, therefore, inform clients at the outset that there are limitations to their confidentiality. School counselors have unique confidentiality issues with the students they counsel (Trice-Black et al., 2013) because of their role and their responsibilities to parents and to administrators. It is important to discuss these limitations thoroughly and to identify foreseeable situations in which confidentiality must be breached (ACA, 2014, §B.1.d.). Counselors may be hesitant to explain the exceptions to confidentiality to new or prospective clients for fear that clients will feel constrained in discussing their problems. However, some researchers have found very little evidence that explaining in detail the limits of confidentiality actually inhibits client disclosures. Others have concluded that the advantages of informing clients about limits outweigh any disadvantages in terms of inhibited disclosure (Baird & Rupert, 1987; Muehleman et al., 1985).

There are numerous exceptions to confidentiality (Kåmpf, 2009; Knapp, 2017). We have identified at least 15 types of situations in which compromising a client's confidentiality might be permissible or required. Because most of these exceptions to confidentiality have both a legal and an ethical basis, we will explore them in more detail after we have discussed the legal counterpart to confidentiality: privileged communication.

PRIVILEGED COMMUNICATION

At the beginning of this chapter, we noted that privacy is a broad concept that provides the underpinnings for both confidentiality and privileged communication. The right to privacy is guaranteed to all citizens by the Fourth Amendment to the U.S. Constitution, which prohibits government searches without warrants. This privacy right is not absolute, however. When the interests of society outweigh the individual's right to privacy, the privacy right is compromised in the interest of preserving a stable societal structure.

Basically, privileged communication means that a judge cannot order information that has been recognized by law as privileged to be revealed in court. The concept of withholding any relevant evidence is antagonistic to the entire system of justice in the United States because legal procedures demand that all evidence relevant to a case be presented to a judge or jury. The legal system also demands that the opposing side in a court case have access to the evidence before the trial takes

place through a process called *discovery*. As a counselor, you will surely want to guarantee that the information that clients give you will be kept confidential. The idea that information and secrets revealed in a counseling session might someday be disclosed in court is very unsettling for counselors. However, the idea that a judge or jury might have to decide the outcome of a court case without the benefit of essential privileged information is very unsettling to judges.

A good question to ask, then, is "Why did the idea of privileged communication emerge in the U.S. legal system in the first place?" According to Slovenko (1998), the evidence presented in a court determines the quality of a trial in a court of law. Despite the strong belief that all evidence should be available to the judge or jury who will decide the outcome of a case, federal and state legislators have realized that the requirement to make available all evidence compromises many important interactions. Legislators have been convinced that, without a guarantee of absolute privacy for conversations between citizens and certain professionals, it would be impossible for professionals to provide necessary assistance to those who seek their help. As a result, statutes have been enacted that specifically exempt certain conversations between citizens and professionals from the general rule in law that all relevant evidence will be presented in a court case. These laws are called *privileged communication statutes*.

Origins of Privileged Communication

According to Shuman and Weiner (1987), the first reference to courts recognizing a legal privilege between professionals and the citizens they served was in early Roman law. This privilege was based on the duty of servants (attorneys) to their masters (clients) rather than on the modern principle of a right to privacy.

Under English common law, the foundation on which the legal system in the United States is based, there was no need for privileged communication. In early England, truth during trials was determined by various modes, including witnesses swearing to tell the truth, defendants being put to a physical endurance test, or individuals who were disagreeing engaging in a battle (Shuman & Weiner, 1987). Even when witnesses were used in trials, they also served as the jurors and relied on information they had gathered as members of the community. Individuals who were not jurors but volunteered information were viewed as meddlers and could be sued for interfering with the legal process.

About 1450, English equity courts began recognizing subpoenas for nonparty witnesses and started using them in law courts in 1562 (Wigmore, 1961). Yet few communications were privileged under English common law. Other than government secrets, the common law recognized as privileged only attorney–client and husband–wife relationships (Knapp & VandeCreek, 1987).

Once the English legal system allowed judges to compel witnesses to testify at trials, a question arose as to whether certain information obtained by professionals should be excluded from testimony for one reason or another. Wigmore (1961), the leading authority on evidence, described four requirements for a relationship to be privileged under the law:

1. The communications must originate in a *confidence* that they will not be disclosed.
2. This element of *confidentiality must be essential* to the full and satisfactory maintenance of the relationship between the parties.
3. The *relationship* must be one that, in the opinion of the community, ought to be sedulously *fostered*.
4. The *injury* to the relationship that disclosure of the communications would cause must be *greater than the benefit* gained for the correct disposal of the litigation.

State and federal legislatures have used these requirements as criteria in considering whether to grant privilege to relationships between citizens and various categories of professionals. The question of which professional relationships meet these criteria and, therefore, should be accorded privilege has been a source of controversy that continues to this day. Some legal scholars have criticized all statutes that have been passed by legislatures granting privilege to relationships with professionals (Chafee, 1943; Curd, 1938; McCormick et al., 2006; Morgan, 1943; Wigmore, 1961). The attorney–client relationship was the first professional relationship to be recognized under English common law, during the reign of Queen Elizabeth I. Currently in the United States, the attorney–client privilege is universally recognized (McKinsey & Burke, 2015).

Physicians, on the other hand, have been less successful at obtaining legal privilege with their patients. According to Shuman and Weiner (1987), most legal scholars agree that the concept of a physician–patient privilege fails Wigmore's test. Almost every jurisdiction in the United States now has privileged communication statutes for physician–patient interactions. However, many of the statutes include a multitude of exceptions, including criminal cases, workers' compensation proceedings, will contests, cases where the condition for which treatment or diagnosis was sought is raised by the patient to support a claim or defense, or cases in which the parent–child relationship is at issue.

According to Knapp and VandeCreek (1987), the clergy–communicant privilege is unique because it has its roots in common law, but it is now protected by statute. These statutes were passed to ensure that confessions by penitents to priests would be absolutely confidential. The clergy, in persuading legislators to pass such statutes, stressed that there was a substantial societal benefit in assuring citizens that their confessions would never be repeated by priests or by members of the clergy outside the confessional. The clergy argued that unless citizens were absolutely convinced that their confessions would remain confidential, they would not confess their sins, which could cause serious disruption to the framework of society. In a few states, despite privilege laws for clergy, they are required to report suspected abuse (O'Reilly & Chalmers, 2014). They asserted that the need for privacy in the confessional far outweighed the need for all evidence to be presented in a trial. The notion of extending the clergy–communicant privilege to include counseling has been criticized by Knapp and VandeCreek (1987). They argue that the privilege for recipients of mental health services should apply only when citizens have relationships with professionals with specialized training in counseling or psychotherapy, and that only a few specially trained pastoral counselors have such qualifications.

McCreary (2011) has explored the unusual situation in which a clergy member was also a licensed professional counselor (LPC) and was providing mental health services. In a Texas case (*Westbook v. Penley*, 2007), a minister disclosed to his congregation private information about a female client's extramarital affair that was revealed in a counseling relationship with the minister (it is standard practice in this church to discuss personal issues brought to the minister with the entire congregation). The Texas court determined that the minister's action was church related, and that a civil court should not interfere, despite the fact that the minister was also an LPC and would not have been allowed to reveal the private information of his client if he had not also been her minister. In this particular case, the court decided that the role of minister outweighed the role of LPC when the two roles were in conflict.

It is very important for counselors who have other roles to avoid trying to assume both roles at the same time. Counselors with dual roles should be clear in their own minds which role they are performing at a given point and communicate effectively their role with the persons they are helping.

Once citizens had been granted privileged communication with attorneys, physicians, and clergy, a variety of other professionals, including mental health professionals, began arguing successfully for privileged communication statutes for the benefit of their clients. Remember, though, that any exception to the general rule that all evidence will be presented in court is compromised by privileged communication statutes. Each time such a statute is passed, the legislators have to be convinced that making an exception to the rule is vital to the well-being of society, and that an individual citizen's need for privacy outweighs the need for evidence in court cases.

The earlier scholars who argued for a mental health professional–client privilege (Cottle, 1956; Fisher, 1964; Guttmacher & Weihofen, 1952; Heller, 1957; Slovenko, 1960; Slovenko & Usdin, 1961) did not base their arguments on the physician–patient privilege. Instead, they contended that citizens who sought the services of mental health professionals required more privacy than patients needed with physicians (Knapp & VandeCreek, 1987). Because of physicians' status in U.S. society, you might think that physician–patient privilege could be considered more important than mental health professional–client privilege. However, some legal scholars (Guttmacher & Weihofen, 1952; Louisell & Sinclair, 1971; Slovenko, 1960; Wigmore, 1961) have concluded that it is more important to protect the privacy of clients of mental health professionals than it is to protect the privacy of physicians' patients because of the unique nature of the psychotherapeutic relationship.

Before World War II, no state had a privileged communication statute for mental health professionals, although some psychiatrists were covered under physician–patient privilege statutes (Knapp & VandeCreek, 1987). State legislative bodies meet annually, changing statutes each time they meet, and sections affecting privileged communication statutes might be found in obscure places in state statute volumes. Therefore, it is impossible to give a completely accurate picture of privileged communication statutes for counselors and other mental health professionals. However, it is true that all 50 states and the District of Columbia have enacted some type of privileged communication statute for mental health professionals and their clients (*Jaffee v. Redmond*, 1996).

Statutes making relationships between mental health professionals and their clients privileged vary substantially from state to state in their language and construction. States have taken four approaches in formulating privilege laws for relationships between clients and their mental health providers (Knapp & VandeCreek, 1987). Some privilege statutes are modeled on attorney–client privilege laws. Others take the approach of the proposed, but rejected, Rule 504 of the Federal Rules of Evidence (28 U.S. Code §504). This rule has advantages over the attorney–client privilege laws because the proposed rules specify the extent and limitations of the privilege with great precision. A third approach provides for a privilege but allows judicial discretion in its waiver. The last group of statutes is idiosyncratic and does not follow any special pattern.

The Rationale for Privileged Communication in Counseling Relationships

Over the past few decades, counselors have been working to convince state legislators to enact privileged communication laws that protect their relationships with clients. Many of the arguments used to justify passage of these statutes were accepted and repeated by the U.S. Supreme Court (Remley et al., 1997) when *Jaffee v. Redmond et al.* (1996) was decided in favor of the existence of such a privilege under the Federal Rules of Evidence.

Counselors have argued that society benefits when individuals seek counseling to help them lead more productive lives as citizens. Mental health professionals have argued that because clients must reveal personal secrets that could be embarrassing, sensitive, or against their best interests, clients must be assured that the content of their counseling sessions will not be revealed without

their permission. Society benefits when clients receive counseling that allows them to be more independent and productive, and these benefits outweigh the negative consequences of some evidence being privileged and, therefore, unavailable in court cases.

However, after an extensive review and analysis of empirical research studies related to confidentiality and privileged communication in the mental health professions, Shuman and Weiner (1987) concluded that although confidentiality is important in therapeutic relationships, privilege is not. Among their more interesting findings were (a) few laypersons know whether privilege exists in their states, (b) many mental health professionals indicate that clients are reassured by their disclosure or threat of disclosure of potential harm to self or others, and (c) judges do not find privilege a great impediment to finding the truth at trials. The first two findings argue against privileged communication statutes that protect relationships with mental health professionals, whereas the last one would seem to support the existence of such statutes.

Another argument used to get counselor privileged communication statutes passed is that the evidence lost to courts because of privilege statutes would never have been produced in the first place if the statutes did not exist. Clients would not go to counselors and reveal their secrets if they did not have the protection of legal privilege. As a result, society does not lose important evidence in lawsuits by granting privilege to counselor–client relationships because the evidence would never have come into existence if there were no privilege.

Psychiatrists, psychologists, and social workers have been successful in their efforts to have statutes enacted that grant privileged communication to their relationships with clients. Counselors have argued that their services are similar to those of other mental health professionals and that they are as deserving of the protection.

Privileged communication statutes vary, ranging from some that protect counselor–client relationships to the fullest extent allowed by law to others that are quite weak. Professional counselors in jurisdictions where there are no counselor–client privileged communication statutes, or where existing ones are weak, are continuing in their efforts to get such laws passed or strengthened. Auerbach (2006) has pointed out that clients who seek mental health services in a particular state have varying degrees of privacy, depending on the exact language found in multiple statutes that grant privilege of clients with psychiatrists, licensed counselors, licensed psychologists, licensed social workers, and other licensed mental health professionals. Auerbach has argued that states need to reform their privileged communication statutes, and that clients should have the same privacy protection, no matter which category of mental health professional provides services.

Privileged communication statutes are not always limited to interactions between clients and licensed professional counselors. In some states, school counselors, substance abuse counselors, or other designated categories of counselors have privilege. On the other hand, if a category of mental health professionals is not listed in a privileged communication statute, courts deny privilege to the relationships of clients with those professionals (Kaplan & Zirkel, 2017; Moberly, 2007). You must investigate the statutes of the state in which you practice to determine whether your interactions with clients enjoy privilege and to determine exactly what *privilege* means in your state.

If you practice in a state that does not offer statutory privilege, you should inform your clients that you will keep confidential the content of your counseling sessions with them, but that one of the exceptions would be if a judge orders you to disclose information. You must include the same exception even if you practice in a state that does include a counselor–client privileged communication statute, but it is much less likely that a judge will order disclosure in a state with such a statute.

Occasionally, counselors will be involved in court cases heard in federal rather than state courts. The U.S. Supreme Court ruling in *Jaffee v. Redmond et al.* (1996) has interpreted the Federal Rules of Evidence to mean that there is a licensed psychotherapist–patient privilege in some cases

heard in federal courts (Remley et al., 1997). In the *Jaffee* case, the psychotherapist was a social worker. It seems probable, but not guaranteed, that a similar privilege would be acknowledged for licensed counselor–client relationships in future cases. In a review of psychotherapist–patient privilege decisions in federal courts, Boumil et al. (2012) concluded that judges usually do not grant privilege to relationships that do not have statutory privilege, but they might determine that the value of confidentiality in a relationship outweighs the need for confidential information in a particular case. The same lack of uniformity in applying the law could be said to describe privileged communication statutes and case law in state courts as well.

Counselors struggle with deciding the exact language to use when writing disclosure statements for clients or when informing clients orally regarding the confidentiality and privilege that might exist in the relationship. On one hand, counselors want to fully inform clients of the exceptions that exist to confidentiality and privilege so that clients will be able to give truly informed consent to entering the counseling relationship (see Chapter 4 under Informed Consent). On the other hand, counselors do not want to be so technical and detailed in describing exceptions to confidentiality and privilege that clients become confused or lose confidence in the privacy of the counseling relationship. Suggested disclosure statements regarding confidentiality and privilege are included in Appendix B.

Asserting the Privilege

A statutory privilege belongs to clients rather than to counselors. If you or your records are subpoenaed or if you are asked during a legal proceeding to disclose privileged information, it is up to your client to assert the privilege so that you will not have to disclose the information (Boumil et al., 2012). However, sometimes the client cannot be located or is not present when counselors are asked to disclose privileged information. In these circumstances, the counselor has an obligation to assert the privilege on behalf of the client. In our opinion, counselors should secure the advice of an attorney if they are put in the position of asserting a client's privilege because legal procedures and questions regarding privilege are quite technical.

5-3 The Case of Abdul

Abdul is an elementary school counselor. He has been counseling a brother and a sister who are in the first and third grades, related to the separation of their parents six months ago. The parents are going through a divorce. The children are living with their mother, and Abdul has regular contact with her. Abdul has never met the father of the children. This morning, Abdul was handed a subpoena issued by the attorney for the father of the children by a person paid to deliver the subpoena to Abdul at his school. The subpoena states that Abdul is ordered to provide copies of all notes he has regarding counseling with the two children and must appear at a deposition in the attorney's office during the school day in two days. Abdul asks the school social worker how he should proceed, and the social worker tells him she has no idea because she has never gotten such a subpoena.

- Should Abdul send copies of his counseling records to the attorney?
- Should Adul appear at the deposition?
- How should Abdul handle this situation?

Discussion: Abdul should not consult with colleagues about a subpoena. Instead, he should talk with his principal; explain that he understands the father may have a right to information

about his counseling sessions with the father's children, but that the mother may have privacy rights as well; and ask the principal to obtain legal advice so that Abdul will know how to proceed. It is the principal's obligation either to consult with an attorney who provides legal advice to the school and relay information from the attorney to Abdul, or to allow Abdul to consult directly with the attorney.

Because subpoenas have legal authority, and because they sometimes are invalid due to privileged communication statutes or laws regarding how subpoenas are served, an attorney's advice is necessary. Abdul could be held in contempt of court if he fails to comply with a valid subpoena, and he could violate the rights of the mother or the children if he complies with an invalid subpoena. Abdul should ask any questions he has regarding complying or not with the subpoena, but ultimately, he should follow the advice he receives from the attorney for the school. If the mother of the children has an attorney, the attorney for the mother will receive a copy of the subpoena that Abdul received from the father's attorney.

Responding to Subpoenas

Subpoenas are legal documents that might require counselors to give a written response to a written list of questions; produce copies of records; appear for a deposition, court hearing, or trial; or appear and bring their records with them. Subpoenas are official court documents and cannot be ignored. Counselors who do not respond appropriately to subpoenas can be held in contempt of court and fined or jailed until they comply. Unless you deal with subpoenas on a regular basis in your work, you should obtain legal advice before responding to a subpoena. Methods of obtaining legal advice are discussed in Chapter 1.

An attorney will review a subpoena for you and will advise you how to respond. If you must appear at a deposition, hearing, or trial as a result of a subpoena, you should ask your attorney to prepare you for what to expect, to advise you about how you should conduct yourself, and if possible, to go with you so you can receive advice throughout the proceeding. More information on responding to subpoenas is contained in Chapter 6.

Suits for Disclosure

When counseling relationships are privileged under state or federal laws, clients have a right to expect counselors to keep information from their sessions private. With or without the privilege, counselors have a legal duty to maintain the privacy of their clients. Clients enter the counseling relationship with assurances of privacy and with very high expectations that information they disclose will not be revealed.

If a counselor discloses confidential information and the disclosure does not qualify as one of the exceptions to confidentiality and privilege, a client could sue the counselor for malpractice. The client would be required to prove the elements of a malpractice suit, as explained in Chapter 8. Clients who could show that they were harmed by the counselor's disclosure might prevail in such lawsuits and collect damages from the counselor. In addition,

clients could complain to licensure or certification boards or to employers if they believe a counselor has disclosed confidential information without their permission. Thus, it is crucial that you understand the exceptions to confidentiality and privileged communication and know how to apply these exceptions in your practice.

EXCEPTIONS TO CONFIDENTIALITY AND PRIVILEGED COMMUNICATION

The general rule that counselors must keep their clients' disclosures confidential is easy to understand and follow. Exceptions to this general rule occur frequently in the course of counseling, however, and counselors must understand these exceptions and be able to apply them to real situations (ACA, 2014, §B.2.). After reviewing counselor privileged communication statutes in all states, Glosoff et al. (2000) concluded that exceptions are numerous and extremely varied. As a result, it is very important for counselors to know the content of the privileged communication statute that exists in their state (if there is one) and the specific exceptions that are included. Glosoff et al. categorized exceptions into nine areas and listed those areas from the most to the least frequently cited in state statutes. Complaints filed include instances when:

- A counselor and client have had a dispute about another matter (such as billing or disagreements);
- A client is litigating their mental condition in a lawsuit;
- A client might be a danger to self or others;
- A report of child abuse has to be filed;
- A client is contemplating committing a serious crime that might harm others;
- A client is ordered to have a psychological examination by a judge;
- A legal proceeding is held to determine if a client needs to be involuntarily hospitalized;
- A client has been a victim in a crime; and
- A client has been accused of harming a vulnerable adult.

To facilitate understanding, we have organized the exceptions into clusters according to the purposes they serve. First, we discuss client waiver of the privilege or right to privacy and exceptions that deal with sharing information with subordinates or other professionals to improve the quality of services. Next, we turn to exceptions that involve protecting clients or others who may be in danger. Then we look at ways that confidentiality and privileged communication are different when working with multiple clients as opposed to individuals, and with minor clients as opposed to adults. Finally, we note some exceptions that are legally mandated.

Client Waiver of the Privilege

Confidentiality and privilege belong to clients, not to counselors. As a result, clients can waive their privacy. This occurs most often when clients explicitly ask counselors to give information regarding the counseling relationship to third parties. Usually, clients who waive the privilege have an understanding that their secrets will be revealed. In some circumstances, however, clients may unknowingly waive the privilege by their actions. For example, clients might implicitly waive the privilege by filing a lawsuit seeking damages for emotional distress caused by an accident or by filing a malpractice lawsuit against a mental health professional, as discussed later in

this section on exceptions. In *Cynthia B. v. New Rochelle Hospital* (1982), the court found that the client (rather than the mental health professional) had responsibility for any embarrassment or inconvenience that resulted from the disclosure of privileged mental health information in a lawsuit she had filed.

Death of the Client

According to McCormick et al. (2006), the common law doctrine is that privilege does not end with a person's death. In some states, statutes specify how privilege is controlled after an individual dies. When there is no statutory language dealing with privilege and the death of the holder, then the common law practice of allowing a legal representative of the deceased person to assert the privilege generally is followed (Knapp & VandeCreek, 1987). An executor of an estate is not always recognized by a court as a deceased person's legal representative for the purposes of privilege (*Boling v. Superior Court*, 1980). In *Swidler & Berlin v. United States* (1998), the U.S. Supreme Court held that even after a client has died, communications with his lawyer that were privileged when he was alive continue to be legally privileged after his death (Kramer, 1999). If you believe that a deceased client's privilege must be asserted or waived, you should contact his or her family members, probate attorney, or executor to determine whether a legal representative is available to deal with the matter. The ACA *Code of Ethics* states simply that counselors must protect the confidentiality of deceased clients according to legal requirements and documented preferences of the client (ACA, 2014, §B.3.f.).

Sharing Information with Subordinates or Fellow Professionals

In some situations, the *umbrella* of confidentiality can be extended to cover others who assist or work with the counselor in providing services to clients. Although these situations do not involve a breach of the client's confidentiality, they do constitute exceptions to the general rule that only the counselor and client are privy to information shared in sessions. Sharing information with others to provide the best possible services to clients is permissible when (a) clerical or other assistants handle confidential information, (b) counselors consult with colleagues or experts, (c) counselors are working under supervision, and (d) other professionals are involved in coordinating client care.

CLERICAL OR OTHER ASSISTANTS MAY HANDLE CONFIDENTIAL INFORMATION It is routine business practice for many types of professionals, including physicians and attorneys as well as counselors, to employ clerical assistants and other subordinates who handle confidential client information. Some state statutes that grant privilege to counselor–client relationships may also specifically extend the privilege to communications when assistants are present or when assistants see privileged information, but such specific language is rare. Knapp and VandeCreek (1987) have concluded that common law principles most likely would extend privilege to unprivileged assistants who are involved in activities that further the treatment of the client.

Counselors should be aware, however, that they are ethically responsible for any breach of confidentiality by someone who assists them. The *Code of Ethics* alerts counselors to "make every effort to ensure that privacy and confidentiality of clients are maintained by subordinates, including employees, supervisees, students, clerical assistants, and volunteers" (ACA, 2014, §B.3.a.). Counselors may also be held legally responsible for breaches of confidentiality by their subordinates. Under the general legal principle of *respondeat superior*, employers are held responsible for

the acts of their employees. Counselors, then, can be held accountable for assistants who breach confidentiality through the doctrine of vicarious liability. It is important for counselors to impress on their employees the importance of keeping information confidential and to supervise them in such a way as to ensure that they are complying with this requirement.

Counselors can take precautions to help ensure that their subordinates understand both the importance of confidentiality and the procedures for maintaining it. Counselors are obligated to determine whether individuals they may hire are trustworthy. Counselors could be held accountable for hiring persons notorious for disregarding the privacy of others if these persons later disclose confidential client information. We suggest that counselors take an inventory periodically of everyone who may handle confidential information about their clients. This might include answering service personnel, an office receptionist, clerical assistants, billing service staff, counselor interns or supervisees, and paraprofessionals. The counselor can conduct training sessions for these employee groups to ensure that they understand the importance of confidentiality and know how the counselor wants them to handle confidential material. The federal HIPAA law (see Chapter 6) requires that employees be trained to protect the privacy of clients. Therefore, training sessions that are held should be documented.

COUNSELORS MAY CONSULT WITH COLLEAGUES OR EXPERTS Counselors are encouraged to consult with peers or experts whenever they have questions about their ethical obligations or professional practice (ACA, 2014, §C.2.e.). If the questions have to do with clinical practice, counselors should seek consultation from a practitioner who has experience or expertise in the particular area of concern. If an ethical question is involved, it is a good idea to consult a fellow mental health professional who has expertise in ethics. In a particularly perplexing ethical dilemma that involves a difficult judgment call, it is wise to seek more than one opinion.

Consultation usually does not need to involve a breach of confidentiality because most consultations can be managed without revealing the identity of the client. The ACA *Code of Ethics* (2014) gives some guidance in this respect. Standard B.7.a. states that "Written and oral reports present only data germane to the purposes of the consultation, and every effort is made to protect client identity and to avoid undue invasion of privacy." Standard B.7.b. cautions that, during consultations, counselors should not disclose confidential information that could lead to the identification of a client. Nonetheless, nearly 30% of the counselors in Neukrug and Milliken's (2011) study believed it was ethical to share confidential client information with a colleague. Although every effort should be made to protect confidentiality, it is sometimes impossible to avoid revealing the identity of the client because of the nature of a particular situation or because the identity of the client might be obvious. In these instances, best practice would be for the counselor to obtain client consent, if possible, and the identity of the consultant should be revealed to the client. Of course, in some situations neither step might be possible.

Finally, counselors should choose consultants who have the appropriate expertise for the question at hand. If you have an ethical or clinical question, consult with a professional colleague. If you have a legal question, ask an attorney.

CONFIDENTIAL INFORMATION MAY BE SHARED WHEN THE COUNSELOR IS WORKING UNDER SUPERVISION While you are a counselor in training or when you are gaining post–master's degree experience as a counselor in preparation for licensure, your work with actual clients will be supervised by experienced professionals. During your practicum, internship, and other field experiences, university supervisors and on-site supervisors will review your counseling sessions with you and

provide you with feedback about your performance. After graduation, if you are planning to obtain a credential such as a license, your initial post–master's degree counseling work will also be under supervision. Experienced counselors may want to provide supervision to beginning professionals or may need to work under supervision while acquiring skills in a new specialty area of practice. Supervision is likely to be a part of your experience throughout your professional career.

Supervision is similar to consultation in that information about clients is shared with other professionals who subscribe to the ethic of confidentiality. However, in supervision, unlike consultation, the client's identity cannot be concealed. Supervisors may observe actual counseling sessions from behind a one-way mirror, review videotapes or audiotapes of sessions, and review case notes and counseling records. Supervisors have the same obligations as you do: They must keep confidential any information revealed in counseling sessions that they are supervising. Nonetheless, your clients have the right to know that you are working under supervision. According to *Informed Consent and Client Rights* (ACA, 2014, §F.1.c.), you have an ethical obligation to inform your clients that their confidentiality is limited because your supervisor will have access to information about them. When you are involved in group supervision, clients have the right to know that a group of your fellow practicum students or interns may also be discussing their cases. The *Code of Ethics* also requires counselors to obtain permission from clients before they observe counseling sessions or review transcripts or videotaped recordings of sessions with their supervisors, faculty, or peers (§B.6.d.).

You may feel some reluctance to divulge to your clients that you are working under supervision, or to explain to them all the circumstances in which information about them might be shared. You may be concerned that clients will have less confidence in your professional abilities, or that they will feel constrained about revealing personal information. These concerns must be set aside in the interest of your clients' right to informed consent. It is best to inform your clients about the limits to confidentiality in a straightforward manner and to do so at the very outset of the counseling relationship. Clients can be told that the purpose of supervision is to help you do a better job and that the focus of the supervision will be on you and your performance rather than on them and their problems. Sometimes clients will be unnerved by the idea of a supervisor, who is a faceless stranger to them, sitting behind the mirror. In these cases, it is a good idea to arrange for the client to meet the supervisor and have the opportunity to express concerns and ask questions directly. In our experience as supervisors, we have seen that clients typically forget their initial concerns once they become engaged in the counseling process. You may be pleasantly surprised to find that this will happen for you, too.

The relationships between counseling students who are in supervision and their clients may not be privileged. As a result, students may be forced to reveal the contents of counseling sessions if ordered to do so by a judge in a legal proceeding. If the relationship between a client and the student's supervisor would be privileged if the supervisor were providing the counseling services, then it is likely that privilege will exist between the client and the counselor–supervisee as well. Usually, under the law of privilege, a privilege is extended to assistants or supervisees of professionals who have statutory privilege with their clients.

OTHER PROFESSIONALS MAY BE INVOLVED IN COORDINATING CLIENT CARE In some settings, such as inpatient units in hospitals or on university campuses (Moss, 2018), treatment teams routinely work together in caring for patients or clients. Although the benefits of coordinating the efforts of various professionals are obvious, as an ethical matter, clients need to be told what information about them is being shared, with whom, and for what purposes (Herlihy & Corey, 2015a). According to §B.3.b. (ACA, 2014), when client care involves a continuing review by a treatment team, the client must be informed of the team's existence and composition, the information being shared, and the purposes of sharing the information.

In numerous other situations, you might want to share confidential client information with fellow professionals in order to ensure coordination or continuity of care. A client might be receiving psychotropic medications from a physician while seeing you for counseling. A client might be seeing you for individual counseling while participating in group counseling with another counselor. Clients might move and later request that you send their records or communicate by telephone with their new counselor. In all these situations, it is ethically appropriate for you to communicate with the other professionals *when the client has given permission for you to do so*. Several standards in the ACA *Code of Ethics* (2014) address these kinds of circumstances. Standards A.3. and B.3. both speak to the need for mental health professionals to work cooperatively. When clients are receiving services from another mental health professional, the *Code* directs counselors to obtain client consent and then to inform the other professional and develop clear agreements to prevent confusion and conflict for the client. Standard B.6.g. (ACA, 2014) requires counselors to obtain written permission from clients to disclose or transfer records to legitimate third parties and reminds counselors that they are required to work to ensure that receivers of their records are sensitive to the client's confidentiality. HIPAA requires that client permission to give information to third parties be obtained in writing and that counselors who transfer records notify the receiving third parties that the information being transferred is confidential and should be protected. Fisher (2008) suggested that counselors mark any records that contain identifiable client information as confidential and not to be released. Also, it is critical that clients be informed of the consequences of disclosing their private information to other parties, especially when they have received diagnoses (Kress et al., 2013).

From a legal perspective, situations in which counselors function in treatment teams or coordinate client care are similar to situations in which counselors' assistants or supervisors learn confidential information about clients. If the relationship between a counselor and a client is privileged, then it is likely that the privilege will continue to exist when confidential information is discussed in treatment teams or with other professionals when coordinating client care.

Protecting Someone Who May Be in Danger

Situations sometimes arise that require counselors to breach confidentiality in order to warn or protect someone who may be in danger. Some of these situations also involve a duty to report the dangerous situation to responsible authorities. These three types of duties—to warn, to protect, and to report—constitute some of the most stressful situations you will encounter as a practicing counselor.

Because these exceptions require that you make a deliberate decision to breach a client's confidentiality, there must be a compelling justification for doing so. Winston (1991) suggested that breaching a confidence can be justified by the concept of vulnerability. A duty to protect from harm arises when someone is especially dependent on others or is in some way vulnerable to the choices and actions of others. Persons in a vulnerable position are unable to avoid risk of harm on their own and are dependent on others to intervene on their behalf. When counselors, through their confidential relations with clients, learn that a vulnerable person may be at risk of harm, they have a duty to act to prevent the harm. This is a higher duty than the duty to maintain confidentiality.

COUNSELORS MUST TAKE ACTION WHEN THEY SUSPECT ABUSE OR NEGLECT OF CHILDREN OR OTHER PERSONS PRESUMED TO HAVE LIMITED ABILITY TO CARE FOR THEMSELVES Counselors have both an ethical and a legal duty to disclose confidential information when such disclosure is required to protect clients or identified others from serious and foreseeable harm (ACA, 2014, §B.2.a.). Duties to protect children and vulnerable adults from abuse are discussed at length in Chapter 11.

COUNSELORS MUST TAKE ACTION TO PROTECT CLIENTS WHO POSE A DANGER TO THEMSELVES When counselors determine that clients may be suicidal, they must do something to prevent the clients from harming themselves (ACA, 2014, §B.2.a.). A leading cause of malpractice lawsuits against counselors is a charge by surviving family members that counselors were negligent and therefore committed malpractice if their clients die from suicide or harm themselves while attempting suicide. This duty to protect your clients from suicide attempts is explained and discussed in Chapter 8.

COUNSELORS MUST TAKE ACTION WHEN A CLIENT POSES A DANGER TO OTHERS When counselors believe their clients may harm someone else, even when such information is communicated by someone other than the client (Edwards, 2006), counselors must take steps to prevent harm to the person who is in danger (ACA, 2014, §B.2.a.). This duty, like the duty to prevent clients from harming themselves, could lead to a charge of negligence or malpractice. The duty to protect others who may be harmed is explained and discussed in Chapter 8.

COUNSELORS MUST DETERMINE WHETHER TO BREACH CONFIDENTIALITY WHEN A CLIENT HAS A COMMUNICABLE AND LIFE-THREATENING DISEASE AND THE CLIENT'S BEHAVIOR IS PUTTING OTHERS AT RISK This exception to confidentiality is the same as the exception to protect others if you believe your client may harm other people (ACA, 2014, §B.2.c.). This exception also is reviewed in Chapter 8.

Counseling Multiple Clients

Unique confidentiality problems arise when working with multiple clients. Generally, when your client and one or more additional persons are in the room, confidentiality is compromised and privilege usually is legally waived. Although you can make your own pledge not to disclose certain information, you cannot guarantee the behavior of others, such as group participants or family members. Despite the fact that others in the room might not keep a client's secrets, a counselor's responsibility is not diminished by the presence of additional persons in a counseling session.

CONFIDENTIALITY CANNOT BE GUARANTEED IN GROUP COUNSELING Confidentiality is a particularly difficult ethical issue in group counseling. Although confidentiality certainly is as important in group counseling as it is in individual counseling, it is difficult to enforce, and counselors should not offer guarantees. It appears that this caution may not be well understood—37% of the ACA members surveyed by Neukrug and Milliken (2011) believed it was ethical to guarantee confidentiality for group members. Group counselors frequently encounter problems with maintaining confidentiality among group members, who are not bound by the same professional standards. They may inadvertently breach confidentiality or yield to the temptation to discuss group experiences with family or friends. The ACA *Code of Ethics* (2014) gives some guidance for dealing with confidentiality in groups in §B.4.a., which states that "counselors clearly explain the importance and parameters of confidentiality for the specific group." A significant portion of Association for Specialists in Group Work's (ASGW) Guiding Principles for Group Work (2021) is devoted to confidentiality among group members (Thomas & Pender, 2008). This is an especially helpful resource with which you will want to familiarize yourself.

A few states specifically provide for privilege to extend to group counseling situations, but most privileged communication statutes are silent on this issue (Knapp & VandeCreek, 1987). It is important, then, that you read the privileged communication statute in your state (if one exists for counseling relationships) to determine whether group counseling sessions are privileged. In states where

group counseling situations are not mentioned, at least one court has found privilege to exist (*State v. Andring*, 1984). As a result of the *Andring* case, it is possible that group counseling sessions would be found by courts to be privileged even if statutes do not specify that privilege exists for group counseling clients. Confidentiality dilemmas related to group counseling are discussed further in Chapter 12.

CONFIDENTIALITY CANNOT BE GUARANTEED IN COUPLES OR FAMILY COUNSELING When the focus of counseling shifts from the individual to the family system, new ethical issues arise, and existing ones become more complicated. Counselors who provide services for couples and families encounter some unique confidentiality concerns, such as how to deal with family secrets. For example, consider the situation in the following case example.

5-4 The Case of Haruto

Bob and Carla, a husband and wife, are seeing Haruto for marriage counseling in Zoom sessions that Haruto does not record. Generally, when Haruto provides counseling to couples, he sees each partner alone for one or more sessions because he believes counseling progresses better that way. Bob shared during one of his individual sessions with Haruto that he is having an affair with another woman and that Carla doesn't know it. Bob doesn't plan to tell Carla, and he insists that Haruto not tell Carla either.

- How should Haruto respond to Bob's insistence?
- How might this situation have been avoided?

Discussion: According to the *Code of Ethics*, in couples and family counseling, "Counselors seek agreement and document in writing such agreement among all involved parties regarding the confidentiality of information. In the absence of an agreement to the contrary, the couple or family is considered to be the client" (ACA, 2014, §B.4.b.). From the wording of the standard, it appears the agreement could be oral or written, but if it is oral, the agreement should be documented in the counselor's notes. Therefore, Haruto should have obtained agreement from Bob and Carla before their counseling sessions began, and documented their agreement in writing, regarding the confidentiality of information, especially because Haruto generally sees each partner in individual sessions. Because Haruto takes that approach to couples counseling, it is very important to establish with the couple how information that is shared individually will be handled. Each partner should know whether or not information shared with the counselor without the other partner being present will be told to the partner who is not present. In this situation, because the *Code of Ethics* states that in the absence of a written agreement the couple "is considered to be the client," Haruto will have to insist that Bob disclose the affair to Carla if Haruto is to continue counseling the couple. Haruto could refer the couple to another counselor.

Haruto might have avoided the problem in several ways. He could have counseled the couple together only, and not individually. He could have told Bob and Carla at the outset that he would not keep any secret that would interfere with the goals or process of counseling, and if either of them objected to disclosing the secret, he would not be able to continue the counseling relationship. Haruto could have included an explanation of his policy on family secrets in his written disclosure statement. In addition, to comply with the ACA *Code of Ethics*, he should have gotten a written, signed agreement from the couple regarding how confidential information would be handled during their couples counseling.

This case example points out the importance of informed consent procedures. You must make clear at the outset to both members of a couple the limitations of confidentiality and how you will handle confidentiality issues, obtain an agreement with the couple, and document the agreement in writing (ACA, 2014, §B.4.b.). In an analysis of the problem of *family secrets*, the ethical code of the International Association of Marriage and Family Counselors (IAMFC, 2017) does not address the issue of family secrets between partners specifically, but instead says that the limits of confidentiality must be agreed upon by clients and their counselor and that information learned by counselors in individual counseling sessions may not be repeated to other family members without the client's permission. This frequently occurring problem is discussed further in Chapter 12.

Counseling couples or families can also lead to very difficult legal questions regarding confidentiality and privilege. Under general legal principles, any statutory privilege that exists between a counselor and a client is waived if there is a third-party present when information is being disclosed. However, privilege laws in some states specifically cover relationships between counselors and married couples or between counselors and family members (Knapp & VandeCreek, 1987). In these states, it is clear that a counseling relationship will be privileged despite specific additional persons being present during the counseling session. In states without such statutes, it is unclear whether an individual who is being counseled with a spouse or other family members present could assert the privilege. Appellate court decisions are split as to whether a privilege that otherwise would exist by statute would be extended to sessions that include a married couple (*Clausen v. Clausen*, 1983; *Ellis v. Ellis*, 1971; *Hospital and Community Psychiatry*, 1979; *Sims v. State*, 1984; *Wichansky v. Wichansky*, 1973; *Yaron v. Yaron*, 1975).

A very difficult dilemma for counselors is posed when one spouse demands that the counselor not reveal information related in a marital counseling session and the other spouse demands that the counsel or reveal it. Situations like this can arise when a married couple enters counseling because of marital problems and then later engages in divorce proceedings.

The spouse who does not want the contents of the counseling sessions revealed should assert the privilege through his or her attorney. Of course, a counselor never wants to reveal in court private information that was related in a counseling session (ACA, 2014, §B.1.c.). Counselors generally will cooperate with the spouse and the attorney as arguments are made to maintain the privilege of the client who does not want the private information revealed. Counselors are quite vulnerable legally in these situations because revealing privileged information inappropriately could lead to a malpractice lawsuit or complaints to the licensure or certification boards of counselors. On the other hand, refusing to disclose information that is not privileged could lead to being held in contempt of court by a judge. If you find yourself in a situation like this and are unsure of your legal responsibilities, you should consult your own attorney or the attorney for the agency in which you are employed, who will advise you regarding your legal obligations and rights. In addition, some national and state counseling associations offer free legal advice to members.

Counseling Minor or Legally Incompetent Clients

WHEN CLIENTS ARE MINOR CHILDREN OR LEGALLY INCOMPETENT, COUNSELORS CANNOT GIVE THE SAME ASSURANCES OF CONFIDENTIALITY AS THEY GIVE OTHER CLIENTS Minors and adults who have been adjudicated incompetent in a court of law do not have a legal right to enter into contracts. Thus, their parents or guardians control their legal rights. As a result, counselors owe legal duties to the parents or guardians of minors or incompetent adults. At the same time, counselors have ethical obligations to the clients themselves (ACA,

2014, §B.5.). This conflict between law and ethics regarding the privacy rights of minors and incompetent adults is discussed in detail in Chapter 11. A number of scholars have expressed concern that minors, particularly adolescents, who could benefit from health care counseling services do not seek them because the minors are not assured of confidentiality (Alderman, 2017; Beeson et al., 2017; Brakman et al., 2018; Leichliter et al., 2017; Taylor et al., 2015). Health services that adolescents might not access due to confidentiality concerns include substance abuse, family planning, gynecological concerns, and sexually transmitted disease services.

Court-Ordered Disclosures

COUNSELORS MUST DISCLOSE CONFIDENTIAL INFORMATION WHEN ORDERED TO DO SO BY A COURT There will be instances when counselors are called to testify in court and their clients ask them not to reveal information shared in counseling sessions. If the relationship is privileged, either the client or the counselor will assert the privilege. In cases where no privilege exists, counselors should ask the court not to require the disclosure and should explain the potential harm that could be done to the counseling relationship if the information is disclosed (ACA, 2014, §B.2.c.). If the judge still requires the disclosure, only essential information should be revealed (§B.2.e.). Counselors who are ordered by a judge to reveal confidential information should not worry that clients may sue them or complain to the licensure or certification boards of counselors for violating their privacy. Complying with a judge's order is a defense to any charge of wrongdoing (Osborne, 2015). Remember, though, that a subpoena that a counselor receives may not be valid. Confidential or privileged information should not be revealed in response to a subpoena until an attorney representing the counselor has advised that course of action. See Chapter 6 for a complete discussion of subpoenas.

Legal Protections for Counselors in Disputes

COUNSELORS MAY REVEAL CONFIDENTIAL INFORMATION WHEN IT IS NECESSARY TO DEFEND THEMSELVES AGAINST CHARGES BROUGHT BY CLIENTS When ethical or legal complaints are filed against counselors by their clients, the concepts of confidentiality or privilege could become problematic for counselors who must defend themselves. It would be odd if a client could claim that a counselor had done something wrong in a counseling relationship, and then the same client could claim that confidentiality or privilege prevented the counselor from explaining the details of the counseling relationship in presenting his or her defense. As a result, the law of privileged communication requires that clients waive their privilege when they bring complaints or malpractice lawsuits against their counselors (Knapp & VandeCreek, 1987).

Other Legal Exceptions

CLIENTS WAIVE THEIR PRIVILEGE WHEN THEY BRING LAWSUITS CLAIMING EMOTIONAL DAMAGE If clients claim emotional damage in a lawsuit, then the law automatically waives their privilege associated with counseling relationships (Knapp & VandeCreek, 1987). It would be unfair to allow individuals to claim in court that they had been emotionally damaged and then to prohibit the counselor who had treated the person for the damage from testifying. The person accused of damaging the client must be given an opportunity in court proceedings to cross-examine the counselor regarding the nature and extent of the damage.

Nonetheless, most courts have held that suits for normal distress or physical injuries arising out of a physical trauma do not automatically waive a plaintiff's right to privilege in counseling relationships (*Ideal Publishing Corp. v. Creative Features*, 1977; *Roberts v. Superior Court*, 1973; *Tylitzki v. Triple X Service, Inc.*, 1970; *Webb v. Quincy City Lines, Inc.*, 1966). Only when individuals bringing lawsuits claim that they were emotionally damaged, and that the damage required them to seek mental health treatment, will their therapeutic privilege be waived.

PRIVILEGE IS GENERALLY WAIVED IN CIVIL COMMITMENT PROCEEDINGS Privilege is usually waived by law for individuals who are being evaluated by a court to determine whether they should be involuntarily committed to a psychiatric hospital. Obviously, the contents of an evaluation conducted for the purpose of rendering an opinion to a court concerning the advisability of a commitment will not be privileged. Evaluators should carefully explain the nature of the interview to individuals who are being evaluated to ensure that the individual does not misconstrue the relationship as involving mental health treatment (ACA, 2014, §E.13.b.; Knapp & VandeCreek, 1987). When a mental health professional seeks a commitment of a client who initiated treatment voluntarily, privilege may also be waived by the client by law (*Commonwealth ex rel. Platt v. Platt*, 1979), although in *People v. Taylor* (1980), a Colorado court refused to waive the privilege. Individuals who are involuntarily committed do not have privileged relationships with their treating mental health professionals at a judicial review of that commitment (*State v. Hungerford*, 1978; *State v. Kupchun*, 1977). The important distinction to remember is that evaluation interviews are different from counseling interviews in terms of privilege. Evaluation interviews conducted to help determine whether a client should be committed to a residential facility are not privileged. Appendix B includes an informed consent form that may be used by counselors who conduct evaluations. See Figure 5-1 for a listing of the exceptions to confidentiality and privileged communication that are summarized in this chapter.

Diversity Considerations in Confidentiality and Privileged Communication

Counselors must be sensitive to the cultural meanings of privacy and confidentiality (ACA, 2014, §B.1.a.). Because U.S. society is becoming more diverse and the international movement in counseling is burgeoning, it is more important than ever before to be aware that some clients may not want their confidentiality upheld in the traditional manner (Tavaokkoli et al., 2015). Confidentiality requirements in counseling have been a reflection of cultural values that emphasize autonomy and respect for individualism (Casas et al., 2010). In collectivist cultures, the individual is seen as less important than the family or group, and clients might not place much importance on their individual privacy, although the privacy of their families or groups might be highly valued (Hoop et al., 2008). In such cases, counselors might need to share confidential information with family members or members of the client's community, if the client so desires. There are cultural differences regarding confidentiality when providing counseling services to minors as well (Olivari et al., 2015).

Maintaining client confidentiality becomes more complicated when the counselor and client do not speak the same language and an interpreter is required. If a family member is used as the interpreter, the counselor should carefully explain the purpose and importance of confidentiality and ask the interpreter to keep all disclosures completely confidential. It is preferable to use a professional interpreter, if one is available, who understands the context of the client's life. The counselor might consider holding a pre-session meeting with the interpreter to review confidentiality, its limits, and how the concept can be communicated to the client (Paone & Malott, 2008). During the initial session, the client should be informed of the limits to confidentiality and privilege that are created by the presence of the interpreter.

FIGURE 5-1 Exceptions to confidentiality and privileged communication

Sharing information with subordinates or fellow professionals is permissible under the following circumstances:

- Clerical or other assistants handle confidential information.
- A counselor consults with colleagues or experts.
- The counselor is working under supervision.
- Other professionals are involved in coordinating client care.

Protecting someone who is in danger may require disclosure of confidential information when the following conditions exist:

- The counselor suspects abuse or neglect of children or other persons presumed to have limited ability to care for themselves.
- A client poses a danger to others.
- A client poses a danger to self (is suicidal).
- A client has a fatal, communicable disease, and the client's behavior is putting others at risk.

Confidentiality is compromised when counseling multiple clients, including the following:

- Group counseling.
- Counseling couples or families.

There are unique confidentiality and privileged communication considerations when working with minor clients:

- Minor clients do not have privilege under the law.
- Privilege belongs to parents or guardians.

Certain exceptions are mandated by law, including the following:

- Disclosure is court ordered.
- Clients file complaints against their counselors.
- Clients claim emotional damage in a lawsuit.
- Civil commitment proceedings are initiated.

Confidentiality is also complicated when counselors provide services to clients during mass trauma events and complex humanitarian emergencies (Tarvydas & Levers, 2017). Counselors may be offering services in public or chaotic situations that make privacy impossible. In these instances, counselors should honor the need for confidentiality to the extent possible by looking for as private a space as is available and by teaching and modeling for other professionals and non-professionals the importance of keeping survivor information confidential.

Summary and Key Points

After reading this chapter, we expect that you have developed a healthy appreciation for the complexities of the ethics of confidentiality and its legal counterpart, privileged communication. Both confidentiality and privileged communication are based on the client's right to privacy. *Confidentiality* is an ethical concept, and *privileged communication* is a legal term.

Counselors encounter dilemmas of confidentiality more frequently than they encounter other types of dilemmas and find them the most difficult to resolve. The considerable amount of information contained in this chapter should be helpful. These dilemmas, nevertheless, will challenge you to use your clinical judgment and your skills in ethical reasoning and in researching the current status of

professional knowledge and the law. We hope we have emphasized how important it is that you do not try to go it alone when you encounter questions about confidentiality and privileged communication, but rather that you seek consultation from colleagues or an attorney.

Following are some of the key points made regarding confidentiality:

- Although research does not clearly support the notion that assurances of privacy are essential to clients' willingness to disclose, counselors view confidentiality as a strong moral force and a fundamental ethical obligation.
- In general, counselors seem to do a very good job of protecting their clients' confidentiality. It is easy for an inadvertent breach to occur, however, and counselors need to be zealous in safeguarding client privacy.
- It could be years before ethical codes, statutes, and case law are created or revised to address the issues of privacy, confidentiality, and privileged communication that have arisen since the COVID-19 pandemic ushered in an era of virtual counseling.
- Generally clients have felt secure about their privacy even though counseling often is taking place digitally since the COVID-19 pandemic began.
- Counselors must take reasonable steps to ensure that digital communications, recordings, and records related to their work with clients are secure.

Privileged communication is a legal concept that is closely related to confidentiality but is different. Privileged communication means that a judge cannot order a counselor to reveal information in court that has been recognized by law as privileged. Because privileged communication is antagonistic to the entire U.S. system of justice, courts and legislatures have been reluctant to extend privilege to relationships between mental health professionals and their clients.

- Except for cases heard in federal courts, privileged communication in counseling is determined by state statutes. It is essential that you learn your state laws regarding privileged communication in counseling relationships.
- Confidentiality and privilege both belong to the client, not to the counselor, and clients can waive their privacy.
- Confidentiality and privilege are not absolute. There are numerous exceptions to confidentiality and privileged communication. You must understand each exception and be able to apply exceptions to real situations you encounter in your practice.

A number of exceptions to confidentiality and privileged communication were discussed in this chapter.

- Confidentiality and privileged communication cease when a client waives those rights or when a client dies.
- It is permissible to share information with subordinates and fellow professionals in order to provide the best possible services to clients.
- Stressful situations for counselors often involve decisions regarding the duties to warn, protect, or report when a client's condition may pose a danger to self or others.
- Confidentiality cannot be guaranteed when counseling couples, families, groups, minors, or adults who have been adjudicated as legally incompetent.
- Other exceptions exist when counselors are court-ordered to disclose information, and when counselors need to defend themselves legally.

Records and Subpoenas

6

LEARNING OBJECTIVES

6.1 Demonstrate an understanding of the purposes for which counselors keep records.

6.2 Identify the various types of records that counselors might keep.

6.3 Identify how the federal regulations included in HIPAA and FERPA are different.

6.4 Explain how counselors should proceed when they receive a subpoena.

FOCUS QUESTIONS

1. Can records that counselors create and are seen only by themselves be subpoenaed?
2. Why is it important to carefully document events in emergency situations?
3. Why should counselors consult with an attorney if they receive a subpoena?

Although records were mentioned in Chapter 5 in relation to counselors' responsibilities regarding confidentiality and privileged communication and are discussed again in Chapter 11 in relation to counseling children, records are such an important (and somewhat complex) area of a counselor's practice that we discuss them in detail in this chapter. Counselors typically receive little training in proper recordkeeping, and they tend to dislike and neglect paperwork (Luepker, 2012; MacCluskie, 2001). Nonetheless, maintaining adequate records is necessary for ethical, legal, and professional reasons. Records are often subject to being subpoenaed, and counselors must be prepared to respond to legal demands for their records.

The Impact of the COVID-19 Pandemic on Records and Subpoenas

The COVID-19 pandemic and the shift to virtual counseling, teaching, and supervision had an impact on recordkeeping in counseling and subpoenas. Prior to the pandemic, counseling records were already primarily being created, stored, and transmitted electronically, but now virtual counseling sessions are being recorded, and other types of counseling records are also being stored and transmitted electronically more often. Court hearings and proceedings that had never been held virtually switched to a virtual format, and, as a result, the nature of subpoenas shifted as well.

As discussed later in this chapter, contemporary counseling records are primarily electronic, although some paper records continue to be created and stored by counselors. The content of counseling records has not changed since the COVID-19 pandemic, but it is critical that counselors understand the proper ways to create, store, and transmit electronically both written and video records. For example, virtual counseling and supervision sessions should be conducted using platforms that encrypt transmissions and are HIPAA-compliant. In Chapter 4, we explained another significant change in that electronic signatures are now legal in the United States, E-signatures are being used in most counseling settings and in courts and have the same legal status as signatures written on paper.

All of the requirements for responding to subpoenas received by counselors discussed in this chapter continue despite the COVID-19 pandemic. However, since the pandemic began, more courts are using electronic communications with parties involved in court proceedings. As a result, counselors may receive subpoenas by email more frequently and may be able to respond officially using electronic signatures. In addition, some official court hearings are being held virtually, and it may be possible for counselors to attend depositions or hearings through telephone or video rather than in-person. When subpoenas are issued, parties are informed whether options other than in-person appearances are possible.

RECORDS

Records are any physical or virtual *recording* made of information related to a counselor's professional practice. When you consider the kinds of records counselors create and maintain, you probably think of the notes that counselors take concerning sessions with clients. These types of records are referred to as *clinical case notes* in this chapter. Other types of records that might come easily to mind are administrative records related to clients such as appointments, billing and payment accounts, copies of correspondence, intake forms, signed disclosure statements, or other routine records that are created in counseling practices. Additional records that are often generated include audio recordings or video recordings of sessions with clients. Since the advent of the COVID-19 pandemic, counselors often provide counseling sessions by video and sometimes record those sessions, with client permission. Recordings of sessions usually are used for supervision purposes, but they might also be created for clients to review or by counselors for the purpose of training other counselors. Unusual records generated in counseling practices might include records of clients logging on to computerized information systems, telephone bills indicating clients' numbers called, computerized records that clients had used for parking or building passes, or video recordings of clients entering or leaving a counseling office. All of these items are records, and the privacy of clients could be compromised if any of these records were not kept confidential.

The entire concept of records has changed during the past few decades. Before recordkeeping became computerized, records were kept on paper and stored in locked file cabinets. And before counseling began to be delivered using video platforms, voice or video recordings were taped, and the physical tapes also were stored in locked cabinets. Today, records kept on paper are rare, and recordings are digital and are stored on computers and on the internet. In the past, paper records could be shredded, and recording on tapes could be physically destroyed. But today, documents or recordings that are digitized might continue to be accessible even after they are deleted. The implications for counseling in our modern world of technology are discussed in Chapter 10.

Although records are a necessary part of a counselor's practice, it is important that counselors balance their need to keep records with their obligation to provide quality counseling services to clients. Counselors who find themselves devoting inordinate or excessive amounts of time to creating and maintaining records probably need to reevaluate how they are spending their professional time and energy.

In today's litigious environment, many administrators and lawyers who represent counseling agencies are requiring counselors to keep voluminous records to protect the agencies in the event a lawsuit is filed. Although counselors are wise to document actions they take when they are fulfilling an ethical or legal obligation (such as writing down how you assessed whether a client might be at risk for suicide and the actions you took; or how you attempted to protect an intended victim from a client's threat of harm), it is not appropriate for counselors to neglect the counseling services they provide to clients in order to produce excessive amounts of records that are self-protective (Kennedy, 2003). Counselors, their administrators, and the lawyers who advise them must balance the need to protect themselves in the event of legal claims and their duty to provide quality counseling services. It is important to note that it is possible to keep adequate records *and* provide quality counseling services.

Purposes of Records

Counseling records are created for a number of reasons (Corey et al., 2024). Some of these reasons benefit clients, whereas others benefit counselors or their employers.

One of the first standards in the ACA *Code of Ethics* (2014) states that counselors must maintain the documentation necessary for rendering professional services to their clients (ACA, 2014, §A.1.b.). From an ethical perspective, this is the primary purpose of keeping records—to assist you in providing clients with the best possible counseling services. Unless you have perfect recall, you will find that the clinical case notes you have recorded will help to refresh your memory prior to sessions. These notes will allow you to review the client's progress toward goals that you and the client have jointly determined, to know when and in what circumstances important decisions and actions were taken, and to maintain an overall perspective on the content and process of the entire counseling relationship.

Clinical case notes benefit the client because they allow the counselor to summarize each interaction with the client and record plans for future sessions. The work of counselors can be more consistent, precise, and systematic when accurate and thorough notes are kept.

Keeping good records allows counselors to stay on track by documenting which kinds of treatments were undertaken and why. It is helpful to counselors, their clients, and the therapeutic process when counselors work to maintain a clear conceptualization of counseling goals and the clients' progress toward those goals. Good records can assist counselors in helping clients measure their change and growth. Counselors can use their notes to reference and affirm significant turning points and successes in their clients' work.

Clinical case notes can be particularly important when a client decides to resume counseling with you after an extended break. It would be a daunting task to try to remember details of a client's life and concerns after months or even years have passed, during which so many other clients' lives and concerns have consumed your attention. If you can use your notes to refresh your recollections about former issues and events that occurred during the counseling process, you will take a significant step toward reconnecting with your returning client.

Continuity of care is a consideration as well when a client is referred from one mental health professional to another. Records are used to transfer the information that one health care provider

has about a client's condition to another health care provider. Health care records create a history of a client's diagnosis, treatment, and recovery. Health care providers can thus render better services when they have access to information regarding a client's past that is more thorough and accurate than a client's oral recall.

Write notes with enough clarity that another helping professional, by learning what you did and did not do to help the client, can provide continuation of counseling. The receiving counselor can get attuned to the meaningful aspects of a client's work if records clearly describe goals, treatment strategies, and progress that was made. If you are the receiving counselor, you will particularly appreciate such records.

Counselors also keep clinical case notes for their own benefit. Counselors document any steps they have taken in an emergency or critical situation. Later, if counselors are accused of not having provided competent services, the records serve as evidence of their thinking, and the notes document any actions taken. Without adequate documentation, there is no clear record of the course and progress of counseling.

In our opinion, before you write any type of note or create a record, you should carefully consider *why* you are taking that action. Many counseling offices or agencies routinely create records without giving any thought to the simple question of *why* the records are being kept. It makes no sense for a counselor who is keeping clinical case notes for the sole purpose of refreshing her memory to write her notes in the same way that the counselor in the local hospital writes notes on a patient's chart. In a hospital, physicians, nurses, physical therapists, counselors, and others read a patient's chart. Each person who makes an entry on a patient's chart is writing so that other professionals will know what types of services were rendered for the benefit of the patient. If counselors ask themselves why they are writing a note for a client record, who will read the record, and what those who read it will be looking for, there will be a better chance that the record will be appropriate for its purpose.

Counselors also keep a number of business records associated with their counseling practice for their own purposes. Counselors who work within agencies or other facilities that provide counseling services often need to document the number of persons who have been served, the categories of services rendered, and other types of information. Counselors in private practice have to keep business records to justify the tax deductions they take in relation to their counseling business. Regardless of whose purpose is being served in creating records, it is the privacy of clients that dictates how records must be handled.

6-1 The Case of Paolo

Paolo has recently taken his first counseling job in the local community mental health center after receiving his master's degree in counseling. During his first week at work, he meets with his section supervisor, who goes over the records the agency keeps for each client. His supervisor shows him an intake form that is 12 pages long. The form requires the counselor to collect all types of personal information on the client, including a thorough medical, health, and social history, and detailed information regarding the client's education, work background, and current job. A diagnosis and treatment plan form requires the counselor to enter a diagnosis from the *Diagnostic and Statistical Manual*, 5th edition, text rev. (American Psychiatric Association, 2022), and at least five measurable and behavioral goals for counseling. Paolo's supervisor explains that after each counseling session, Paolo must note in the client's folder a summary

of issues discussed in the session, the client's progress toward each of the counseling goals listed, and what Paolo plans to do in the next counseling session. The supervisor suggests that Paolo write about three pages of notes for each client after each session. His supervisor also says there are more forms to be completed after clients have their last session, but they will go over those forms later. When Paolo asks his supervisor why so much information is collected for each client, his supervisor replies in a patronizing tone, "This is the way all professional counseling agencies keep records. Weren't you taught recordkeeping in your master's degree program?"

- How should Paolo respond to his supervisor's training session?
- What kinds of records do you think would be reasonable for a community mental health agency to keep on each client?

Discussion: Because Paolo has accepted employment at the agency, he should do his best to understand exactly what types of records he is expected to keep and to create those kinds of records. After he has been at the agency for a reasonable period of time, he might suggest to his supervisor and other staff members that recordkeeping procedures be reviewed to determine if records are being kept appropriately, given the goals and objectives of the agency's counseling program.

Regarding the types of information that would be practical for a community mental health agency to keep, intake information might be limited to basics, such as contact information and current medications or health problems. A diagnosis should be rendered only if a disorder exists and is required by third-party payers, and only after the client has consented to the diagnosis becoming a part of the record. In addition, treatment plans and goals for counseling should be flexible, and the number of goals should be determined individually for each client. In clinical case notes, only information needed to refresh the counselor's memory should be written down. There is no need to summarize the content of sessions unless a counselor is documenting what occurred in a session for self-protection.

Ethical Standards Related to Records

Standard A.1.b. of the ACA *Code of Ethics* (2014) states that counselors include in their records "sufficient and timely documentation to facilitate the delivery and continuity of services." This standard acknowledges that *continuity of services* is an important reason for keeping records. Standard A.1.b. also requires that counseling records document client progress and services provided and that errors in client records be corrected according to agency or institutional policies.

Section B.6. of the *Code of Ethics* (ACA, 2014) includes nine standards that specifically address counseling records. Standard B.6.b. addresses the importance of keeping records in a secure location. Counselors must strive to keep records in a place that is not accessible to unauthorized persons. Counselors are responsible for ensuring the safety and confidentiality of any counseling records they create, maintain, transfer, or destroy. This caveat includes records that are written, taped, computerized, or stored in any other medium. Keeping records digitally is discussed in depth in Chapter 10, which focuses on technology. In Chapter 5, we discussed at some length how easy it can be for a counselor to inadvertently breach a client's confidentiality. That is as pertinent to counseling records as it is to other aspects of the counseling relationship, as is evident in the case of Vanessa, presented here.

6-2 The Case of Vanessa

Vanessa is a licensed professional counselor (LPC) in private practice with a group that includes three other counselors. One afternoon at 1:00 p.m., she has had a client cancellation and is using the time to work at her computer writing clinical case notes from her morning sessions. Jasmine, the counselor whose office is next door, pokes her head in the door and asks Vanessa for a case consultation. Vanessa and Jasmine go into Jasmine's office and close the door for privacy. Unbeknownst to Vanessa, her 2:00 p.m. client arrives early, sees that her office door is open, and goes in. The client reads the clinical case notes that Vanessa left on her computer screen.

- What actions can counselors take to ensure that a client's records are not seen by anyone not authorized to see them?
- What steps must counselors take to preserve client privacy when they keep case notes on a computer?

Discussion: Counselors should use the following guidelines to prevent unfortunate situations like the one just described:

- In your office, be extra careful not to leave records out in the open, where other people can see them. Always put records away, even if you leave your desk for only a few minutes. If you are working from home, as counselors often did during the COVID-19 pandemic, the same cautions apply.
- Use a secure password for accessing client records on your computer. Always close a secure file when you leave the room.
- If your computer is networked, be sure your client records are not accessible to others.
- Keep in mind that documents or videos that are digital might still be able to be recovered even after they have been deleted.
- Position your computer so that others in your office cannot easily see what you have on your screen.

Standards B.6.c. and B.6.d. (ACA, 2014) address the issue of informed consent and require counselors to obtain client permission before they electronically record or observe sessions. Recording or observing sessions, common practices in supervision, are discussed in more detail later in this chapter.

Standard B.6.e. (ACA, 2014) alerts us to the fact that we have an ethical obligation to provide clients with access to their records, unless the records contain information that might be misleading or detrimental to the clients. There is a presumption that clients have a right to see records that counselors keep related to the counseling services that the clients have received. Standard B.6.e. states, "Counselors [should] limit the access of clients to their records, or portions of their records, only when there is compelling evidence that such access would cause harm to the client." You will see in the section on the Health Insurance Portability and Accountability Act (HIPAA) that only in rare circumstances can a mental health professional refuse to allow clients to see their records. See 45 CFR 164.524(a)(2)-(4). This standard also provides guidance on the tricky situation that occurs when clients have been seen in the context of group or family counseling. In situations involving multiple clients, counselors should limit access to those parts of the records that include confidential information

related to another client. When clients are given access to their counseling records, counselors should provide them with assistance and consultation in interpreting the records (§B.6.f.).

Standard B.6.g. (ACA, 2014) requires counselors to obtain written permission from clients to disclose or transfer records to legitimate third parties unless exceptions to confidentiality exist. Interestingly, this standard also makes counselors responsible for taking steps to ensure that receivers of counseling records are sensitive to the confidential nature of those records. It is not possible for a counselor to guarantee anyone else's behavior, including that of a fellow mental health professional who receives records. However, steps can be taken to promote professionally appropriate transfers of records. When transferring records, either on paper or electronically, include a cover letter that explains the confidential nature of any transferred records, mark each page *confidential*, and include a statement on the records that the copy of the records is not to be transferred to any third party.

Standard B.6.h. (ACA, 2014) provides guidelines to counselors regarding storage and disposal of counseling records after a counseling relationship has ended. Although this standard could be interpreted to imply that counselors must keep counseling records indefinitely, our suggestion is that some records be kept and others destroyed when it is clear that a client will no longer receive services from a counselor. The issue of whether to keep or destroy counseling records is discussed at length in later sections of this chapter.

Standard B.6.i. (ACA, 2014) imposes on counselors the duty to take *reasonable precautions* to protect their clients' privacy if counselors terminate their practice, become incapacitated, or die.

One important client privacy issue is how much information third-party payers (health insurance companies) should be allowed to demand from counselors and how much information counselors should be willing to provide them (MacCluskie, 2001). Welfel (2016) has suggested that before counselors sign a contract with a health insurance organization, they should learn the type, frequency, and extent of patient information the organization requires in order to authorize and review treatment. As an informed consent procedure, of course, this information about the extent their insurance company will have access to their mental health records should be thoroughly discussed with the client. Standard B.3.d. (ACA, 2014) requires counselors to obtain authorization from their clients before disclosing information to third-party payers.

Legal Requirements

Overall, clinical health care records are created to benefit the person who is receiving the services. In fact, legal principles indicate that the contents of the records about a particular client belong to that client, even though the paper or recording instrument belongs to someone else (*Claim of Gerkin*, 1980; *People v. Cohen*, 1979). Records can benefit clients in a number of ways.

The *Code of Ethics* (ACA, 2014) provisions regarding records do not conflict with any legal requirements regarding counseling records. In fact, one ethical standard (§B.6.g.) goes beyond legal requirements and indicates that written permission should be obtained from clients before records are transferred, unless specific exceptions exist. The common law in the United States does not require that counselors obtain written permission from clients to give copies of their records to third parties. However, many agency policies, state statutes, and federal laws (such as the Health Insurance Portability and Accountability Act [HIPAA] and the Family Educational Rights and Privacy Act [FERPA]) have such written permission requirements. In addition, it is generally advisable that counselors, in order to protect themselves, obtain written authority from clients before transferring records to third parties. If a client later denies that such permission was given or is not available to say whether oral permission was granted, the counselor will have

a document as evidence that permission was obtained. Counselors should use forms created by the agency in which they work when any client requests records. If the agency does not have a standard form, or if a counselor in private practice does not have such a form, the sample form provided in Appendix C can be used.

There is no common law principle, nor are there any general federal or state statutes, requiring counselors to keep clinical case notes. On the other hand, the *Code of Ethics* (ACA, 2014), in §A.1.b., implies that ethical counselors must keep clinical case notes that document services rendered to clients and client progress. Counselors do keep various types of records for a number of legally related reasons. For example, counselors in private practice must keep administrative records regarding their expenses and income for federal and state income tax purposes. As is discussed later in this chapter, some specific federal statutes require counselors to handle records in particular ways if counselors work in settings in which these statutes apply. In addition, lawyers always advise counselors to carefully document important actions they take so that they (counselors) can protect themselves if their actions or judgments are later questioned in a lawsuit.

Confidentiality and Privileged Communication Requirements

All the points made in Chapter 5 regarding the confidentiality and privileged communication responsibilities of counselors related to the privacy of clients apply to records to the same degree as they apply to oral comments. However, because records have physical properties, whereas *information* may not, it is important to carefully consider the responsibility of counselors to handle records appropriately.

TYPES OF RECORDS KEPT BY COUNSELORS

A common complaint of personnel in agencies that hire counselors and internship site supervisors is that counselor education programs do not adequately prepare counselors regarding recordkeeping (Luepker, 2012; MacCluskie, 2001). General principles regarding creating, maintaining, and transferring records are taught in counseling master's-level degree programs. The settings where counseling services are rendered vary substantially, and as a result, the types of records created, the approach to recordkeeping, record storage practices, and specific information requirements in records vary substantially from setting to setting. For example, the records kept by counselors employed in in-patient hospital settings are very different from records kept by counselors employed in high schools (Shea et al., 2018). In addition, many agencies that employ counselors have purchased or developed recordkeeping software that requires training for those who use the system. It would be impossible for counselor educators to prepare entry-level counselors to keep records for every setting in which they might become employed. Instead, orientation for new counselor interns or counselor employees needs to include specific training in recordkeeping requirements in that particular setting.

The ACA *Code of Ethics* (2014) provides only limited information about the types of records that must be kept; rather, the *Code* allows each counselor or each entity that employs counselors to determine which types of clinical case notes are needed to render professional services. Many recordkeeping requirements are imposed on counselors by laws, regulations, and procedures within a work setting, rather than being required by the *Code*.

Standards A.1.b., A.1.c., A.2.a., B.6.a., and B.6.g. provide guidance to counselors regarding the types of records they must keep.

Standards A.1.b. and B.6.a. both state that counselors maintain documentation "necessary for rendering professional services." This section also indicates that records must "include sufficient and timely documentation to facilitate the delivery and continuity of services." Records must accurately reflect client progress and the services that counselors have provided to clients. Any amendments to records must be made according to agency or institutional policies. From these standards, it is clear that counselors are expected to keep some type of record for all clients and that records should be kept in such a way that another counselor could take over the care of the client, if necessary.

Standard A.1.c. requires that counselors develop counseling plans with their clients and revise the plans as necessary. Although the *Code* does not specifically require that such plans be in writing, that expectation can be assumed.

Standard A.2.a. states that "counselors have an obligation to review in writing and verbally with clients the rights and responsibilities of both counselors and clients." This standard suggests that there is an ethical obligation to have a written record of informed consent for counseling relationships and that the document must be reviewed orally with clients. The content of these written disclosure statements is detailed in Chapter 4.

Standard B.6.g. requires a written record of permission to transfer records.

It probably would be helpful for counselors-in-training or beginning counselors to be provided with more information about the recordkeeping responsibilities that will be a part of most of the jobs they will hold. However, it would not be wise for the profession, the ACA *Code of Ethics*, or counseling faculty members to tell counselors the types of records they must keep, the contents of such records, and the ways in which those records should be created. It is best for counselors and the agencies that employ counselors to determine the kinds of records they need, based on their position and the kinds of clinical notes they need to create in order to be effective practitioners. Keeping in mind that there will be variations depending on a counselor's style and the requirements imposed by employment settings, we offer some general guidelines regarding three types of records: administrative records, recordings, and clinical case notes.

ADMINISTRATIVE RECORDS All settings that provide counseling services keep administrative records. The type of agency and external pressures determine how many and what types of records are kept. When counselors accept employment in an agency, school, hospital, or other setting, they agree to conform to agency rules and procedures, which include recordkeeping requirements.

Administrative records are any types of records that would not be considered recordings of sessions or clinical case notes. Administrative records include appointment books or electronic appointment systems, billing and payment accounts, copies of correspondence, signed informed consent documents, client permission to release information, intake forms, and other routine papers that are created as a result of providing counseling services to clients. In most businesses, records of these types are not considered confidential. Yet the very fact that someone has applied for or received counseling services is a private matter. Therefore, business records that reveal the identity of individuals who have had contact with the agency must be treated as confidential and must be protected in the same way that clinical case notes or counseling session recordings are protected.

RECORDINGS Clinical supervision of counseling services takes place in some form in most settings. Agencies often accept practicum or internship students who must record their sessions with clients for the purposes of clinical supervision. Mental health professionals, after they have received their graduate degrees, usually are required to practice under clinical supervision for 2 to 3 years before they can become licensed. In addition, many practicing counselors continue supervision throughout their careers and might record sessions for supervision purposes.

Counseling sessions could be recorded when counselors want clients to listen to or view their own behavior or interpersonal interactions during sessions, although this is not a frequent practice. Counselors who ask clients to listen to or view their own behavior during sessions should emphasize to their clients that these recordings would compromise their privacy if the clients did not keep them secure or if they failed to erase them after using them. Counselors also should make sure clients know that until these recordings are erased or destroyed, they would be available for subpoena in some lawsuits that could arise. We suggest that counselors who give recordings of sessions to their clients for any purpose have their clients sign a form to ensure that they understand the problems that might arise and their responsibility for their own privacy protection.

A number of procedures must be followed when recordings of client sessions are created. Clients must know that recordings are being made and must agree to the process. The ACA *Code of Ethics* (2014) requires that consent of clients be obtained prior to recording sessions (§B.6.c.). The consent of clients must be *informed* in that clients must understand the reason the recordings are being made and who will have access to them. It would be a violation of privacy if sessions were recorded without a client's knowledge, or if consent were given and then the recordings were used for purposes not agreed to or were accessed by persons unknown to the client. Recordings, once they are created, must be kept as secure as any other confidential record is kept. Once the recordings have been used, they must be erased or destroyed. However, counselors must be aware that it is possible that digital documents or recordings might be able to be accessed even after they have been deleted.

Although there is no general legal requirement that the client's permission be obtained in writing, counselors often obtain written permission from clients to record counseling sessions in order to protect themselves. This self-protection would be necessary if a client later claimed that permission had never been given for the recording or that the client did not fully understand how the recording would be used. In addition, many agencies require written permission before client counseling sessions may be recorded. If such policies exist, counselors must comply. Any signed form of this type obtained from a client should be placed in the client's folder and kept until the recording is deleted or destroyed. If you wish to obtain a client's permission to record a session for supervision and your agency does not have a standard form to use, a form is provided in Appendix D.

6-3 The Case of Téa

Téa counsels children in the counseling lab at the university where she is working on her master's degree in counseling. Before she begins her first session with Ana, an 8-year-old, Téa explains to Ana that there is a camera in the corner of the room and that their sessions will be recorded so that Téa's instructor can review their counseling sessions. Ana says she understands and doesn't mind that their sessions will be recorded. After their session is over, on the drive back home, Ana tells her mother that the counseling session was recorded. Ana's mother calls the director of the counseling lab and tells the director that Téa recorded the counseling session with Ana without Ana's knowledge and without the mother's knowledge or permission.

- What should Téa have done to prevent this problem?
- Why is it important to obtain written permission to record counseling sessions, even though verbal permission is adequate from a legal perspective?

Discussion: Because Ana is a minor, it was necessary for Téa to obtain at least verbal permission from Ana's mother to record the counseling session. It would have been best practice for Téa to obtain written permission to record from both Ana and her mother. Even though written permission to record is not legally required, it is best to get permission in writing so that you have proof that permission was obtained.

CLINICAL CASE NOTES Clinical case notes that counselors keep regarding their sessions with clients are the records that concern counselors and clients most. These notes sometimes contain specific details clients have told counselors, counselors' clinical impressions of clients, and generally very sensitive and personal information about clients.

A problem with clinical case notes is that counselors often think of them as being personal notes to themselves that will be seen only by them in the future. However, the reality of clinical case notes is much different, and we have found that even seasoned counselors are sometimes surprised to learn how many people have a right to access their clinical case notes. Counselors must be aware of how these notes are viewed by clients, agencies, and the law.

We often have heard the suggestion that counselors simply should not take clinical case notes so that they will not have to worry about the notes being subpoenaed or being seen at a later time by the client or any other person. This is not sound advice. In the first place, the ACA *Code of Ethics* (2014) specifically states that ethical counselors do keep records (§A1.b.). In addition, it seems to be general practice today for mental health professionals, no matter what their credentials or orientation, to routinely create and use clinical case notes in their practices. If counselors were to fail to create these notes, it is likely that they would be accused of unprofessional practice and probably would be asked to explain how they could render quality counseling services without taking clinical case notes (Anderson & Bertram, 2012). Second, not having clinical case notes would not keep a counselor from having to reveal confidential information orally from memory if someone had a legal right to that information. In addition, counselors should take the clinical notes they need in order to function effectively as professionals. Counselors should not feel inhibited about creating clinical case notes just because the notes may later be seen by clients themselves or by third parties.

ASSUME NOTES WILL BE READ Counselors never know whether others will read the clinical case notes they create. Thus, counselors should assume that notes they write will become public information at some later date. Counselors who make this assumption will be very cautious in deciding what to include in their notes. At the same time, counselors should write whatever they need to render quality counseling services to their clients.

Situations in which others appropriately and legally read counselors' clinical case notes are discussed in the following sections. It is important for you to remember the following sobering information when you are writing clinical case notes:

- Your clients have a legal right to review the notes and to obtain copies of them.
- Your clients have a legal right to demand that you transfer copies of those clinical case notes to other professionals, including other mental health professionals, attorneys, physicians, and even accountants.
- Your clients can subpoena the clinical case notes when they are involved in litigation.

- Other parties can legally subpoena the clinical case notes when involved in litigation situations involving the client, sometimes over the client's objection and even when the records are privileged, if legal exceptions to privilege exist.
- The legal representatives of deceased clients, in most states, have the same rights to clinical case notes as the clients had when they were alive.
- Clinical case notes sometimes do become public information and get published in the media.

APPROPRIATE CONTENT OF CLINICAL CASE NOTES Comparatively less instruction occurs in counseling graduate programs regarding the purposes and the actual creation of clinical case notes than in other professions such as medicine, nursing, and social work. In these other professions a heavy emphasis is placed on how to write health records, what information should and should not be included, and proper ways of creating such clinical notes. Most health records are passed from one professional to another, and professionals rely on each other to record information so it can be used in the care and treatment of a patient or client. Counseling clinical case notes, on the other hand, generally are written by a counselor for the counselor's own use. Sometimes, however, others read these records, and counselors even offer such records to prove they acted professionally. The American Psychological Association (2007), Luepker (2012), Mitchell (2007), and Merlone (2005) have provided suggestions for keeping records. Some guidelines for counseling case notes are included in Appendix E. We offer some recommendations here regarding the creation of clinical case notes to assist counselors in their practices.

You should never lose sight of the purposes of clinical case notes. There are two basic reasons to keep notes: (a) to provide quality counseling services to clients and (b) to document decisions you have made and actions you have taken as a counselor. Both of these purposes are legitimate and reasonable.

6-4 The Case of Jennifer

Jennifer is a high school counselor. She has just completed a brief counseling session with April, a 14-year-old student with whom Jennifer talks occasionally. April has had trouble adjusting to her new school after moving to the area mid-year. During the session, April told Jennifer that a new female friend recently told April that she didn't want to be April's friend anymore because April spent time with girls the new friend did not like. April said to Jennifer, "This is horrible. I feel like killing myself." After talking through the situation with April, Jennifer asked April an appropriate series of questions to determine whether April was serious about the idea of killing herself. Jennifer's questions included asking directly, "Are you really thinking seriously about killing yourself?" and asking whether April had ever attempted suicide. Based on April's responses, Jennifer decided that April was not suicidal and invited April to return to see Jennifer if she ever did think seriously about suicide. Immediately after April left her office, Jennifer had to go to a staff meeting with the other counselors in the school and after the meeting had appointments with two sets of parents of other students. Jennifer didn't have time to write any notes regarding her session with April until the next morning.

- Why is it important to document what the counselor did and said after counseling sessions like this one?

- When should Jennifer have written the notes that documented what occurred in this session?
- Why is it important to document sessions for self-protection immediately after counseling sessions end?

Discussion: Jennifer should have taken the time to document what she and April had said during the session immediately after the session ended, even if it meant going late to her staff meeting. In the event that Jennifer's conclusion that April was not at risk for suicide was inaccurate and April later made a suicide attempt, Jennifer's notes would demonstrate that Jennifer said and did what a reasonable counselor would have done: completed an appropriate suicide potential assessment and appropriately concluded that April was not at risk. Notes that are meant to document what occurred to demonstrate a counselor took appropriate actions in a counseling session must be written immediately after the session ends, or within a very short time frame. Notes written long after a session ends sometimes are not credible because memories about details fade as time passes. It is impossible to predict whether a client actually will attempt suicide, but it is possible to perform suicide potential assessments and to make reasonable conclusions based on clients' responses to questions that are asked. Counselors must make suicide assessments when appropriate, but counselors are not held accountable for predicting whether a client will attempt suicide if they can demonstrate that they made a thorough assessment.

RECORD NOTES IMMEDIATELY It is critical that you take notes either during sessions or immediately after. It is inadvisable to write clinical case notes long after the counseling session occurs. The whole purpose of the notes is to record on paper what your memory will not hold. If you wait too long, you will be relying on your memory, which is what you are trying to avoid.

Your own personal style should dictate whether you take notes during a counseling session or after it is over. Both approaches have advantages and disadvantages. Taking notes during sessions allows you to record accurate information immediately without having to rely on your memory. Some counselors can take notes unobtrusively during sessions. Other counselors may have trouble taking notes and listening to clients at the same time. Clients could be distracted or inhibited by your note taking during sessions, if it is not done easily. Some counselors find that waiting until after the session is over is the best way for them to take clinical case notes. We suggest that you try both methods and adopt the approach that works best for you.

As we have emphasized, the first purpose for creating clinical case notes is to render quality counseling services. A very important part of writing clinical case notes is to separate objective information (what was said by anyone or observed by you during the session) from your clinical impressions (hypotheses or conclusions you developed as a result of what was said or observed). Quite often, you will review notes to determine what a client actually said during a session. You must be able to separate what was actually said from what you thought about as a result of what was said. In addition, your notes may be used someday as evidence in a legal proceeding, and being able to determine what was actually said by a client could be critical. A common format for clinical case notes is the use of *SOAP* notes, which include four separate sections:

Subjective: information reported by the client

Objective: results of the counselor's tests and other assessments administered

Assessment: the counselor's impressions generated by the data

Plan: diagnosis and treatment plan, along with any modifications to them

Another format that is similar and is also used frequently is the *DAP*, which is composed of three sections:

Data: objective description of what occurred during the session

Assessment: counselor's interpretations based on the data, in the context of the presenting problem and treatment plan

Plan: what the counselor intends to accomplish in the next session or sessions

DOCUMENTING DECISIONS MADE AND ACTIONS TAKEN The second purpose for keeping clinical case notes is to document decisions you have made or actions you have taken. Such documentation would be important when you determine that a client is a danger to self or others and you take some action to prevent harm. Documentation is also important when you consult with others regarding a client's situation. When you make decisions that clients may not like, you should also document your actions in your clinical case notes. If you decide to terminate counseling over a client's objection, instruct a client to take some action the client is reluctant to take, or impose limits on a client's interactions with you outside of sessions, it is wise to document how and why you did these things and the client's reactions. Wehrman et al. (2010) have provided guidelines for school counselors regarding when they should document for self-protection.

When you document decisions or actions in your case notes, it is to protect yourself in the event that such decisions or actions are questioned later by anyone else (Mitchell, 2007). As a result, it is important to provide as much detailed information as possible. For example, listing the exact time a conversation took place could be vital. When you are talking to people other than your client, it is important to include the person's name, title, and contact information; date and time the conversation took place; and any other information that might be needed later. Recording exact words used by you and others often can be important as well. When documenting to protect yourself, it is essential that you do so immediately after conversations take place. If you write notes hours, days, or weeks after conversations have occurred, such notes will be much less helpful to you if you are trying to prove that you made the decisions or took the actions you are claiming in your notes to have taken.

It would be impossible to write a summary of every action you take as a counselor or to make an audio recording or video of every counseling session. Excessive documentation can also take away valuable time that might better be spent providing counseling services. On the other hand, in some circumstances counselors should document their actions to create a record showing that they have done what they should have done. Guidelines for documenting for self-protection are included in Figure 6-1.

WRITE LEGIBLY Another important part of note taking is to write notes that are legible, at least to you. For notes to be useful in the future, you must be able to read and understand them. If your notes are transferred to clients or third parties at a later date and they cannot read them, you will be called on to explain what you wrote. This could be very inconvenient for you because it could occur in a court after you have been subpoenaed and put under oath to tell the truth. It is much better to write your notes so that others can read them than to have to interpret them later. It is definitely not a good idea to purposefully write notes so that they cannot be read by others or to use symbols that only

FIGURE 6-1 Documentation of records can provide self-protection

Generally, counselors must provide clients with access to their records upon request (ACA, 2014, §B.6.e.). However, if there is *compelling evidence that such access would cause harm to the client*, then counselors can limit clients' access to their records. If counselors do decide to deny client access to their counseling records, then "Counselors document the request of clients and the rationale for withholding some or all of the record in the files of clients" (§B.6.e.).

1. Document circumstances in which your actions or inaction may be later reviewed by an ethics panel, licensure board, or administrator, or within the context of a legal proceeding.
2. Some of the situations in which some level of documentation is called for include the following:
   - Someone accuses a counselor of unethical or illegal behavior.
   - A counselor reports a case of suspected child abuse.
   - A counselor determines that a client is a danger to self.
   - A counselor determines that a client is a danger to others.
   - A client who is being counseled is involved in a legal controversy that could lead to the counselor being forced to testify in court. Such controversies could include but are not limited to counseling a child whose parents are arguing about custody in a divorce case, a husband or wife involved in a contentious divorce case, a couple contemplating a divorce, or a person involved in a personal injury lawsuit.
3. Begin documenting as soon as you determine that the situation is one in which documentation is important.
4. When documenting for self-protection, include as much detail as possible. Include dates, exact times events occurred, and exact words spoken to the degree that those details are remembered. Include only factual information. When documenting, avoid recording your thoughts, diagnoses, and conclusions. If these must be written down, include them in clinical case notes rather than in records kept for documentation. But remember that all notes usually have to be produced if your records are subpoenaed.
5. The best documentation is created very soon after a conversation or event has occurred. Indicate the date and time anything is written, and never backdate anything. In other words, do not imply or state that something was written on an earlier date than when it was actually written.
6. If you realize that you should have begun documentation sooner, write a summary of what happened up to that point in time. Include as much detail as can be remembered, as well as the date and time the summary was written.
7. Maintain a documentation file that includes the originals of notes written to counselors, copies of notes written by counselors to others, copies of relevant papers that counselors cannot keep for themselves, and other papers that might be relevant to the situation.
8. Keep documentation records safe in a locked file drawer or cabinet or in a secure computer file. If counselors agree to provide their files, they should never release their originals, only copies.

you can understand. When clients or third parties try to read such notes, they often become irritated. You can be put in a difficult position because it may appear that you are not cooperating with people who have a legal right to review the notes.

Client Access to Records

It would never occur to most health care providers to have concerns about patients having access to copies of their records. However, mental health care is different from physical health care in many respects. Some mental health care professionals believe it is not appropriate and could even be harmful for patients or clients to have access to information about their diagnoses of mental disorders, treatment approaches being taken, or clinical impressions that have been formulated by the professional.

The legal perspective on health care records is that they are kept for the benefit of the patient, and therefore the patient should have access to them and must receive copies if requested. Further, the patient has the right to demand that one health care provider transfer the records to another health care provider. Mental health care is not seen as different from physical health care concerning patient or client records. It is important, then, for counselors to assume that clients will have access to clinical case notes created, a right to copies of those notes, and a right to request that copies of the notes be transferred to other professionals.

In the event that a client requests a copy of your clinical case notes and you have any reason to think that it would be best if the client did not see them, some practical steps could resolve the matter. You could explain why your client would be better off not seeing your notes, and your client might accept your explanation and withdraw the request. You also could suggest that it might be more helpful for the client or for the person to whom the client wants the records transferred if you were to create a summary of your notes rather than copy the notes themselves. You might explain that the notes were written for your own clinical purposes and that others probably would be more interested in summaries than in day-to-day notes.

If you were to refuse to show a client your clinical case notes regarding treatment or refuse to provide copies for the client or for a third party that the client has designated, the client probably could take legal steps to compel you to comply with the demand. It would appear that the law supports the client's rights to the records (Dickerson, 2022). Access to their mental health records by clients is common practice in most countries (Blease et al., 2021). In the following section, you will find that HIPAA requires counselors to give clients access to their records. There is an exception in which counselors can refuse to provide client access when counselors believe a client would be harmed by the contents of the record, but the exception is narrow, and the assumption is typically that a client should have access.

In some circumstances, your clinical case notes may contain information about other parties involved in sessions in which the client was counseled. Such situations could arise in couples, family, or group counseling. If your clinical case notes contain information that would compromise another person's privacy if the notes were given to a client or to someone designated by the client, you might try to convince your client to withdraw the request, or you might suggest that you provide a summary for your client. If your client insists, however, on having copies of your original records, then you should copy what you have written, eliminate references to others (which is known as *redacting*), and provide your client with the portions of the records that deal only with that client. When counselors provide clients with copies of their counseling records, counselors do not include confidential information related to any other clients (ACA, 2014, §B.6.e.).

FEDERAL LAWS AFFECTING COUNSELING RECORDS

It would be impossible to describe every federal law or regulation that contains requirements for counseling records. The most far-reaching federal law related to mental health records is HIPAA. Most other federal statutes related to mental health records apply only to programs that are funded by federal tax money. Counselors who work in settings that are administered by the federal government or in agencies that receive federal funding should ask their employers whether any federal laws or regulations affect

the counseling records they keep as part of their jobs. Many private agencies as well as state and local government programs receive federal funding. Three federal laws affect a number of counselors: the Health Insurance Portability and Accountability Act (HIPAA), the Family Educational Rights and Privacy Act of 1974 (FERPA), and the Comprehensive Alcohol Abuse and Alcoholism Prevention, Treatment and Rehabilitation Act of 1972. While there are additional federal statutes that affect counseling recods, these three have the most profound effect on recordkeeping for counselors.

Health Insurance Portability and Accountability Act

Public Law 104-191, commonly known as HIPAA, caused a huge stir in the health care industry in the United States when it was first enacted. According to the U.S. Department of Health and Human Services (HHS, 2014), the HIPAA rules protect the privacy of individually identifiable health information.

HIPAA was enacted on August 21, 1996. The HIPAA statute required HHS to issue privacy regulations governing individually identifiable health information if Congress did not enact privacy legislation within 3 years of the passage of HIPAA. After Congress failed to act on time, HHS issued a HIPAA rule, known as the *Privacy Rule*, on December 28, 2000. Final modifications to the rule were published on August 14, 2002. A full text of the rule and modifications are located at 45 CFR Part 160 and Part 164, Subparts A and E, on the agency website.

HIPAA applies only to organizations and individuals who transmit health care information in electronic form in connection with a health care transaction. As it would be very difficult to have a *paper-only* mental health care practice (Blakely et al., 2004), we recommend that all mental health organizations and professionals in private practice comply with the HIPAA requirements (Lawley, 2012; Wilkinson & Reinhardt, 2015).

The Privacy Rule requirements went into effect on April 14, 2003. All records and other individually identifiable health information held or disclosed by a covered entity in any form—whether communicated electronically, on paper, or orally—are covered by the HIPAA Privacy Rule.

Counselors must give clients a clear written explanation of how counselors use, keep, and disclose their health care information. Clients must have access to their records. A process must exist and must be in writing for clients to request amendments to their records. A written history of most disclosures must be available to clients. In addition, counselors must have available for clients to see, if they request it, a set of written privacy procedures that include who has access to protected information, how it will be used within the counseling office, and when the information would or would not be disclosed to others. Under HIPAA, clients must have a means to make inquiries regarding their counseling records or to make complaints regarding the privacy of their records.

Counselors must obtain uncoerced client consent to release information for treatment, payment, and health care operations, purposes, and for nonroutine uses and most non–health care purposes, such as releasing information to financial institutions determining mortgages and other loans or selling mailing lists to interested parties, such as life insurers. In an interesting lawsuit, the plaintiff argued that the fees mental health providers charge to transfer copies of records to third parties are excessive and should not be allowed under HIPAA (Fisher, 2018).

Disclosures of treatment information without specific client authorization are allowed in certain circumstances, including but not limited to quality assurance oversight activities, research, judicial and administrative hearings, limited law enforcement activities, emergency circumstances, and facility patient directories.

One unique provision could be very helpful to counselors when they provide services to clients who need a great deal of family support. If clients are first informed and then given an opportunity to orally object, counselors may give health care information to family members or others assisting in the client's care. Standard B.1.a. of the *Code of Ethics* (ACA, 2014) cautions counselors to maintain sensitivity to cultural meanings of confidentiality and "respect differing views toward disclosure of information." We recommend that counselors who wish to utilize the HIPAA provision allowing private information to be given to family members indicate in their initial client agreement form the intention to communicate with family members when needed and to specifically ask clients orally whether that is acceptable to them. Under this same section in the rules, counselors may provide information to the public in the form of facility directories and to disaster relief organizations, such as the Red Cross.

When clients do give permission to transfer their personal information regarding their counseling, only the minimum amount of information to accomplish the purpose may be transferred. As a result, it is important for counselors to have on their record release form a place for clients to specify the purpose of their request that their records be transferred. Standard B.2.e. of the *Code of Ethics* (ACA, 2014) states that when disclosure of confidential information is required, counselors should reveal only essential information. When clients request that their records be transferred to another health care professional in conjunction with anticipated treatment from that individual, then, according to HIPAA, the full record must be transferred so that the receiving professional can provide the best-quality care.

Under HIPAA, each counseling office must designate a privacy officer. In a private practice, the counselors would also serve as the privacy officers. The privacy officer of an agency must train employees to handle confidential information appropriately, ensure that procedures to protect client privacy are in place and are followed, and ensure that proper forms are used by counselors and other health care personnel.

When counselors transfer records based on clients' written permission, they must take steps to ensure that recipients of the records protect the privacy of the information they have received. We recommend that counselors record and insert statements that the contents are confidential and should not be transferred to any other individuals.

What we have called *clinical case notes* are known as *psychotherapy notes* under HIPAA. Psychotherapy notes that are used only by the psychotherapist, to which no one else has access, are held to a much higher standard of protection. The rationale is that such notes are not a part of the health care record and were never intended to be shared with anyone else. When a client wants to have psychotherapy notes transferred to a third party, the client must specifically request their transfer. A general request for transfer of records is insufficient. Clients must give their specific permission and authorization for the sensitive information contained in psychotherapy notes to be released. Health care plans cannot refuse to provide reimbursement if a client does not agree to release information covered under the psychotherapy notes provision.

Civil penalties for violations of the HIPAA rules are $100 per incident, up to $25,000 per person, per year, per standard. Criminal penalties for providers who knowingly and improperly disclose or obtain health care information include substantial fines and terms of imprisonment.

The Office of Civil Rights received more than 34,000 HIPAA violation complaints in 2021, which was up 25% from the previous year. In 2021, the Office of Civil Rights conducted more than 570 compliance reviews that led to 83% of the entities reviewed having to take corrective action or pay a civil penalty (Pifer, 2023). The extent to which HIPAA protects the privacy of clients is still unclear. One state court has ruled that, under HIPAA rules, a patient who was undergoing review for mandated, court-ordered outpatient treatment would have to be given notice and the opportunity

to object before the patient's records could be provided to a court through a subpoena (Gomez & Knight, 2013; *In the Matter of Miguel M. v. Barron*, 2011).

HIPAA is a federal law, so it applies throughout the United States and overrides state laws that might be more lax. However, state laws that are stricter than HIPAA about protecting consumer health care privacy must also be adhered to if they exist.

Family Educational Rights and Privacy Act

The Family Educational Rights and Privacy Act of 1974 (FERPA), which is sometimes referred to as the *Buckley Amendment*, affects all public educational institutions and any private or parochial educational institution that receives federal funding in one form or another (Doll et al., 2011). Almost all private and church-sponsored educational institutions receive some type of federal funding. The penalty for violating provisions of FERPA is the loss of all federal funding, which could be devastating for many educational institutions. Individuals cannot bring lawsuits under FERPA.

Since the HIPAA rules were enacted, school personnel have wanted to know whether HIPAA requirements for health care records must be followed in educational institutions. In some cases, HIPAA applies in schools, and in other situations, it does not. Under 24 CFT 164.501, health information contained within student educational records that are subject to FERPA is exempt from the requirements of HIPAA. On the other hand, school-based health centers may be subject to HIPAA requirements if such centers are operated by HIPAA-compliant health organizations. Also, school nurses may be subject to HIPAA requirements if they engage in electronic billing. Communications between schools and health care providers may be subject to HIPAA rules as well. Administrators in charge of school counseling programs should consult with school board attorneys if they are unsure whether their school counseling programs must comply with HIPAA rules.

FERPA legislation basically says that minor students' parents (and students themselves who are 18 or older or who are in college) have two rights: (a) to inspect and review their education records and to challenge the contents to ensure the records are not inaccurate or misleading and (b) to have their written authorization obtained before copies of their education records can be transferred to any third party (U.S. Department of Education, 2014).

After students are 18 years old or are attending a postsecondary institution, the legal rights under this legislation are vested in them. However, parents or guardians of dependent students may be given copies of the records of such students without the students' consent. *Dependent students* are defined as children or stepchildren, over half of whose support was received from the taxpayer the previous tax year (Family Educational Rights and Privacy Act of 1974, 26 U.S.C.A. §152). As a result of this definition, most educational institutions give access to student records to parents who can show they claimed their child as a tax dependant the previous year.

Education records are defined in the federal legislation as records kept by educational institutions regarding students. The law specifically exempts any records that "are in the sole possession of the maker" [FERPA, 1997, 20 U.S.C.A. §1232g(a)(B)(i)] from the right to inspect and review them. To qualify under this exemption, counselors in schools and other educational institutions should keep their case notes separate from other records and should not let anyone else have access to those records. Treatment records "made or maintained by a physician, psychiatrist, psychologist, or other recognized professional or paraprofessional" are also exempt [20 U.S.C.A. §1232g(a)(B)(iv)]. As a result of these exemptions, clinical case notes kept by counselors in educational institutions do not have to be shown to students, parents, or guardians under FERPA requirements.

Schools are not required by FERPA to obtain written permission from parents or students to release a student's records to "other schools or school systems in which the student intends to enroll" [FERPA, 1997, 20 U.S.C.A. §1232g(b)(1)(b)]. Nonetheless, when copies of records of transfer students are forwarded to other schools, parents must be notified of the transfer and must be given copies of the transferred records if they desire copies.

These exemptions for access to inspect and review one's records under FERPA do not in any way diminish a person's rights to counselors' records under a subpoena in relation to a lawsuit. A valid subpoena for records must be complied with in all circumstances. When an educational institution intends to release records pursuant to a subpoena, the institution must notify parents and students in advance of the release [FERPA, 1997, 20 U.S.C.A. §1232g(b)(2)(B); *Mattie T. v. Johnston*, 1976; *Rios v. Read*, 1977], although notification is not required if the subpoena orders that the existence of the subpoena not be disclosed [FERPA, 1997, 20 U.S.C.A. §1232g(b)(J)(i–ii)].

An amendment to FERPA (1997) has given permission to release private information without parent or student written permission in an emergency situation. The information must be "necessary to protect the health or safety of the student or other persons" [20 U.S.C.A. §1232g(a)(1)(I)]. This provision would allow counselors to release their records of students to medical personnel or law enforcement authorities without violating FERPA, if they determined that a student was a danger to self or others.

Educational institutions may release to an alleged victim of any violence the results of any disciplinary proceeding conducted against the alleged perpetrator with respect to that specific crime [FERPA, 1997, 20 U.S.C.A. §1232g(b)(6)]. FERPA also allows institutions to include in educational records any disciplinary action taken against students and to disclose such action to school officials in other schools [20 U.S.C.A. §1232g(h)]. However, institutions may not release to newspapers the names of students who have been found guilty of disciplinary proceedings (*United States v. Miami University*, 2002).

No general common law principle requires counselors to obtain *written* authorization from clients to transfer records to third parties. Oral permission is legally adequate, but obtaining written authorization is always recommended, and under FERPA, written authorization is required to transfer records.

Federally Funded Substance Abuse Programs

The federal government funds a multitude of substance abuse prevention and treatment programs. Private and public agencies apply for federal funds to operate many of these programs. Programs that receive federal funds must comply with federal laws related to substance abuse treatment.

A federal statute, the Comprehensive Alcohol Abuse and Alcoholism Prevention, Treatment and Rehabilitation Act of 1972, 1997 (42 U.S.C.A. §290dd-2), declares that records kept by any facility that is "conducted, regulated, or directly or indirectly assisted" by the federal government are confidential. Disclosure of records of individuals receiving substance abuse services (including education, prevention, training, treatment, rehabilitation, or research) is permitted only in a few circumstances. Exceptions to the nondisclosure of records requirement include the following: (a) when the person gives prior written consent, (b) in medical emergencies, (c) for audits or evaluations, and (d) to avert substantial risk of death or serious bodily harm if a court order is secured. Records cannot be used for criminal charges or investigations [42 U.S.C.A. §290dd-2(c)].

However, a counselor's direct observations are not a record and can be used in criminal proceedings against clients or patients (*State v. Brown*, 1985; *State v. Magnuson*, 1984).

The statute related to federal substance abuse programs does not prohibit exchange of information between the military and the Department of Veterans Affairs. The statute also permits the reporting of suspected child abuse and neglect under state laws.

Unlike for FERPA, the violation of this statute or the regulations that implement it can result in criminal charges and a fine. As a result, clients in federally supported substance abuse treatment programs may file complaints against counselors who violate clients' rights to privacy, although individuals whose confidentiality rights are violated cannot file civil lawsuits for damages (*Logan v. District of Columbia*, 1978).

Other Federal Statutes

Additional federal statutes prohibit or limit the disclosure of counseling records for the following clients who are served by federally assisted programs: (a) runaway and homeless youth (42 U.S.C.A. §5731); (b) individuals with sexually transmitted diseases [42 U.S.C.A. §247c(d)(5)]; (c) voluntary clients in federal drug abuse or dependence programs [42 U.S.C.A. §260(d)]; (d) older persons [42 U.S.C.A. §3026(d); 42 U.S.C. §3027(f)(1)]; and (e) victims of violence against women [42 U.S.C.A. §14011(b)(5)]. On the other hand, statutes sometimes expressly limit privacy. A certain federal statute specifically denies confidentiality to individuals who are examined by a physician in civil commitment proceedings due to drug abuse (42 U.S.C.A. §3420).

Handling, Storing, and Destroying Records

Creating counseling records is an important process, but maintaining and ultimately disposing of them are crucial matters for counselors as well. According to the *Code of Ethics* (ACA, 2014), counselors must ensure that records are kept in a secure location and that access to counseling records is limited to authorized persons (§B.6.b.). The privacy of clients and the self-protection of counselors both have an impact on the manner in which records are handled, stored, and destroyed.

Because counseling records are so sensitive, agencies should develop written policies regarding how these records will be handled from day to day. Agencies or practices regulated under HIPAA must have written policies regarding records and must make those policies available for client review. Typically, counselors create and keep their own clinical case notes. These notes should be stored in locked drawers or cabinets, or on a secure site when the records are computerized. In offices with more than one person, an individual should be designated as the records manager. Generally, this person would be a clerical or administrative employee. The records manager should ensure that counseling records are kept secure, checked out and back in properly, stored safely, and, eventually, eliminated according to a set schedule and policy.

Everyone who handles counseling records should make sure that the records are not left unattended in such a manner that unauthorized persons can read them. Care should be taken to promptly return counseling records to their proper place after they have been used.

No general laws dictate how long a particular counseling record must be kept (AHIMA, 2013). However, some federal or state statutes require that some types of records created for certain purposes be kept for a particular period of time. For instance, some state counselor licensure laws dictate

that counselors' records be kept for a particular number of years. Attorneys who advise counseling agencies should be asked to review statutes and regulations to identify any requirements regarding records, including the length of time they must be maintained.

Many agencies have determined that they want to keep their counseling records for a set period of time. Generally, agencies also have rules and guidelines regarding how records should be handled, stored, and eliminated. Counselors should always inquire as to whether the agency in which they work has such requirements. Counselors should follow all agency rules and regulations.

Counseling records are often also affected by agency accreditation rules. Many counseling centers, hospitals, and treatment facilities are accredited by private agencies such as the Joint Commission, the International Association of Counseling Services (IACS), and the Commission on Accreditation of Rehabilitation Facilities (CARF International). These accreditation agencies have specific requirements regarding counseling records, and many require that records be kept for a set period of time. Counselors who work in accredited agencies should ask about recordkeeping requirements.

If no laws, agency rules, or accreditation standards require records to be kept for a specific length of time, then counselors should encourage their agencies to establish a written policy and to destroy counseling records on a periodic basis. There are a number of reasons for not keeping counseling records indefinitely. Practically speaking, it is expensive to keep records locked away and to keep up with logging them in and out. In addition, confidential records that exist could be compromised in some way, whereas confidential records that have been destroyed cannot be seen by unauthorized persons.

It is wise to establish a record-destroying policy stipulating that certain categories of records will be destroyed on a specific date each year unless one of the following applies: (a) there is reason to believe the records may be subpoenaed in a current or future lawsuit; or (b) the records contain documentation of actions taken by counselors that will be kept longer than the usual period of time. There are no guidelines about how to determine whether individuals *believe records may be subpoenaed*. Common sense should dictate this determination. If records are purposefully destroyed when someone knows or reasonably should have known that those records would be subpoenaed later, the person destroying them can be charged with a serious crime. Records documenting actions that counselors have taken to protect themselves probably should be kept longer than other counseling records normally would be kept.

Voluntarily Transferring Records

Clients often request that their counseling records be provided to another professional who is rendering services to them. In addition, professionals often request your clients' counseling records when they begin treating your current or former clients. The ACA *Code of Ethics* (2014) requires that counselors obtain written permission from clients to disclose or transfer copies of their records (§B.6.g.). After clients have requested or given permission in writing that their counseling records be transferred to a third party, counselors have an obligation to comply. A form is provided in Appendix C that can be used when clients wish to have their records transferred.

Counselors should make sure the written request of the client is followed precisely and that only copies of records the client authorizes for transfer, not the originals of records, are actually sent. Also, records located in client files from other professionals or agencies should never be transferred to a third party. If clients want those records sent, they must contact the agency that created the records and authorize that agency to forward them.

The written and signed authorization from the client should be filed in the client's folder. In addition, a notation should be entered in the client's file indicating which records were sent and when, the method of transfer (e.g., U.S. mail, courier service, personal delivery, electronic transfer), to whom they were sent, and the address of the recipient.

SUBPOENAS

Receiving a subpoena can be an intimidating, and even frightening, experience for a counselor (DeCino et al., 2019; Gutheil & Drogin, 2013). Unless counselors deal with subpoenas routinely in their work, this reaction is justified. Dealing with a subpoena places a counselor in the legal arena, which is a system that is adversarial in nature and operates by a different set of rules than those to which counselors are accustomed (Borkosky, 2016). If you should receive a subpoena, the information that follows will help guide you through the process of responding to it.

6-5 The Case of Isabella

Isabella, an elementary school counselor, has been counseling Billy Rosen, a third-grader whose parents are going through a contentious divorce. One afternoon after school, Isabella is working in her office when a gentleman appears at her door. He announces that he has a subpoena for her records that has been issued by Mr. Rosen's attorney. He offers to wait while she makes a copy of the records of her counseling sessions with Billy.

- How should Isabella respond to the gentleman's offer?
- What should Isabella do about the subpoena?

Discussion: Isabella should first inform the gentleman that she will confer with her supervisor to determine how to respond to the subpoena. She should then tell him she will not produce the records until some later time, if the subpoena is valid. Isabella should notify her immediate supervisor that she has received a subpoena and request legal advice regarding her response to it. If her supervisor does not provide her with access to a lawyer, then Isabella should follow the supervisor's directives regarding how to respond to the subpoena.

Subpoenas are legal documents that might require counselors to produce copies of records; appear for a deposition, court hearing, or trial; or appear and bring their records with them (Barsky & Goud, 2024; Childress-Beatty & Koocher, 2013; Cottone & Tarvydas, 2007; Levy et al., 2014). Subpoenas are official court documents and cannot be ignored. Even though subpoenas might result in health care professionals having to turn over records or appear in court for various reasons, counselors must know whether they have a legal obligation to do what a subpoena says.

In Chapter 5, we noted that a court order is one of the exceptions to confidentiality and privileged communication. Court orders might be issued in hearings or trials at which judges are present and give verbal orders, or they might be issued when judges sign written orders in their official capacity. All citizens must obey court orders. The sanctions for not obeying orders issued by judges are imprisonment, fines, or both. The only alternative to obeying a court order is to appeal to a higher court. A lawyer must file such appeals immediately. Although a subpoena is a type of court order, some subpoenas are not valid, so counselors should always consult with their own attorneys before turning over records or appearing at a deposition, hearing, or trial in response to a subpoena.

Discovery in Litigation

Lawyers who are representing parties in lawsuits are members of the bar in the jurisdiction in which they are practicing. Therefore, they are considered official officers of the court. One of the privileges extended to officers of a court is to issue valid subpoenas for information related to cases they are litigating.

One of the concepts in lawsuits of any kind is that attorneys representing the parties are entitled to *discovery*. Discovery is the process whereby attorneys have the right to ask for and receive information relevant to their case before the case is tried. The idea is that there should be no surprises during trials. Instead, before the actual trial begins, each side in a lawsuit should know all the information that will be entered into evidence during the trial.

Subpoenas are used extensively during the discovery or pretrial phase of lawsuits. Attorneys from both sides ask potential witnesses to respond in writing to written questions (interrogatories), to provide copies of records they are entitled to see, and to come to their offices for depositions. In addition, witnesses can be subpoenaed to testify at hearings before the judge presiding over the case, as well as at the trial.

Validity of Subpoenas

If an attorney issues a subpoena to you, your first step is to consult your own lawyer to determine whether the subpoena is valid and whether you must respond to it. Guidelines for obtaining your own legal advice, whether you are an employee or are in private practice, are provided in Chapter 1.

The records of counselors may be subpoenaed, as may counselors themselves for their oral testimony at depositions, hearings, or trials. Also, written questions may be submitted with a demand for written answers. Counselors might also be subpoenaed to appear at a legal proceeding and instructed to bring records with them (Bowerbank, 2006). In the event you receive a subpoena to appear at a legal proceeding but no mention is made of records, do not take records with you unless you are instructed to do so by your own attorney.

An attorney will review a subpoena for you and will advise you regarding the proper manner in which to respond. Attorneys might assist you in a number of ways. For example, they could ask the attorney who issued the subpoena to withdraw it, file a motion to quash the subpoena, or advise you to comply with the subpoena.

Attorneys who are litigating cases may not issue subpoenas for records or for witnesses to testify if the attorneys know they do not have a right to the information. However, because attorneys may not realize that the material may be privileged, you could receive a subpoena that is not enforceable because of privilege (American Psychological Association, 2006; Woody, 2007). Subpoenas could be unenforceable or invalid for other reasons as well. You must therefore obtain legal advice regarding your response to any subpoena you receive. If you refuse to respond to a valid subpoena, you could be held in contempt of court and either imprisoned or fined. On the other hand, if you provide information in response to an invalid or unenforceable subpoena, you could be held legally accountable to a client or former client for any damages suffered.

When you receive a subpoena, you should take action immediately because most subpoenas must be complied with within a fixed number of hours. Many counselors mistakenly believe that subpoenas must be personally handed to them to be valid and enforceable. In fact, in most cases, subpoenas are mailed, left with someone else at your place of work, or delivered to you in some fashion other than being personally handed to you. If you have knowledge—through any means—that a subpoena has been issued to you, you should immediately take the steps listed in the following sections.

Your first step, if you are employed, should be to notify your immediate supervisor that you have received a subpoena and to request legal advice regarding your response to it. If your employer does not provide you with access to a lawyer, then follow the directives of your supervisor. If you are in an independent private practice, then your first step should be to call your own practice attorney.

Your lawyer or supervisor should be informed if you think the information being requested might be privileged. You should also inform your attorney or supervisor if you believe testifying or producing

records regarding the matter would compromise the privacy of a former or current client, even if the relationship was not privileged by law. You should feel free to openly discuss the situation with a lawyer because your relationship with the lawyer is privileged. Often, it is necessary to explain to your lawyer the general concepts of counseling, the nature of your relationship to the parties involved in the lawsuit, and the details regarding clients who are involved or affected by the litigation. You should give complete information to your lawyer and answer any questions regarding the situation.

Once your attorney or supervisor understands the situation and has given you advice, follow the instructions precisely. If you have any doubts or questions, discuss them with your attorney or supervisor, but ultimately you must follow the advice given to you.

Generally, a subpoena received from the attorney of your client or former client will not be problematic because the individual in most cases has waived his or her privilege, if it exists, by calling you as a witness. Nonetheless, seek legal advice to ensure that you are proceeding properly.

Interrogatories

One of the easiest types of subpoenas to deal with involves interrogatories. Included with the subpoena will be a set of written questions with a requirement that you respond to each question in writing. You must sign an oath that your answers are truthful.

If you receive a set of interrogatories and your attorney informs you that you must respond to them, you should discuss each question and your proposed answer with your attorney before you begin writing. After composing your answers and before you submit them, you should ask your attorney to review your answers. Take your attorney's advice about changes or rewording of your answers. Again, if you are an employed counselor and do not have access to an attorney, follow the directives of your immediate supervisor.

Appearances at Proceedings

In the event you are directed by your attorney or supervisor to appear at a legal proceeding, ask that you be told what to expect. School counselors who have been required to appear at court hearings have reported that the experience was stressful (DeCino et al., 2017). The school counselors in this study said that in dealing with subpoenas, they relied on consultation, advice, and support from administrators, lawyers, colleagues, educators, and consultants, but some participants said they were very dissatisfied with the guidance they received from their administrators and school attorneys. The school counselors reported they would say less if they were forced to testify in court again and would talk with the students involved before testifying in court. The study participants indicated that appearing in court and sitting in the witness stand was exceptionally anxiety-inducing and that they wished someone had been with them to advise them through the process.

Ask your attorney or supervisor to explain in detail how such events take place. Also, ask the attorney to tell you what questions you might be asked and request advice on responses you think you should make to the anticipated questions. In addition, ask the attorney to accompany you to the proceeding if possible.

Depositions usually take place in the office of the lawyer who has issued the subpoena. Although the judge will be present at hearings and trials, the judge does not attend depositions. Generally, depositions are attended by the attorney who issued the subpoena, the attorney representing the other party involved in the lawsuit, a court reporter, and the witness being deposed.

It is important to have your own attorney present at your deposition if possible. During a hearing or trial, the judge will rule on controversies and will instruct witnesses whether to answer

questions. With no judge present at depositions, you need your own attorney there to tell you how to proceed and to protect you from being pressured by the other attorneys. In some circumstances, one attorney could tell you that if you do not answer a question, she or he will issue a rule to show cause and require you to appear before a judge to explain your refusal to answer, and tell you that the judge could then hold you in contempt of court and fine or imprison you. The other attorney could then threaten that if you do answer the question, that attorney will sue you for violating his or her client's privacy. When situations like this arise, you need the presence of an attorney who will advise you how to proceed.

Testimony Under Oath

Testimony that you give at any legal proceeding will be *under oath*, which means that you will be required to swear to tell the truth. Once an oath has been administered, false statements usually are considered perjury, which is a serious crime. You might be instructed by your attorney not to answer questions at a deposition, but you should always, in all circumstances, answer questions truthfully when you do respond under oath.

Turning Over Records

In the event that your attorney or supervisor directs you to turn over your records in response to a subpoena, you should follow the advice you have been given. Sometimes attorneys who issue subpoenas will accept summaries of notes rather than the case notes themselves, or other, similar compromises may be needed. Your attorney will negotiate appropriately on your behalf.

You should never forward the originals of your records. Instead, make copies and deliver the copies. Sometimes records of other individuals are mixed in with records that must be turned over. In that situation, sometimes it is permissible to obliterate private information related to other parties. Again, follow the advice of your attorney in this situation.

If your records include records that you have received from other professionals—such as physicians, other mental health professionals, or hospitals—do not copy and forward those records. If the attorney issuing the subpoena wants copies of those records, the individual or entity that created them will have to be subpoenaed.

The *Code of Ethics* (ACA, 2014) states that when counselors receive subpoenas for the counseling records of their clients, counselors must obtain written informed consent from their clients to release the records or, if that is impossible, must "take steps to prohibit the disclosure or have it limited as narrowly as possible due to potential harm to the client or counseling relationship" (§B.2.d.). In our experience, clients who do not wish to have their counseling records exposed rarely agree to sign a written consent document. Usually, clients whose records have been subpoenaed, after being informed by their counselors that a subpoena exists, take appropriate legal steps through their attorneys to have the subpoena quashed. It is the duty of clients to assert their legal privilege if one exists. In the event a client is not available to be informed that his or her counseling records have been subpoenaed and it is believed that the relationship with the client was privileged, the counselor or the counselor's attorney should assert the client's privilege on behalf of the client and should take other steps that are legally available to avoid complying with the subpoena. If the subpoena is determined to be valid, then counselors and their attorneys should do what they can to limit the disclosure of private client information.

Summary and Key Points

This chapter familiarizes you with the complexities of counseling records and recordkeeping. Subpoenas were also addressed because records are often subject to being subpoenaed. With respect to *records*, some key points made in this chapter include:

- With the advent of electronic recordkeeping and virtual counseling services, the entire concept of recordkeeping has changed over the past few decades.
- Counselors keep records for two legitimate reasons: to provide the best-possible quality of services to clients and for their own self-protection.
- Good records can benefit clients in several significant ways. Some benefits to clients include continuity of care when client information is transferred from one mental health professional to another or when a former client returns to counseling after an extended absence; helping clients measure and affirm their growth and change; and the creation of an accurate history of a client's diagnosis, treatment, and recovery.
- Records can also serve to protect counselors. If your decisions are ever challenged, it is essential that you have documented decisions that you have made and actions you have taken.
- Virtual counseling and the use of computers for recordkeeping have had significant impacts on the ways counselors create and maintain records. However, recordkeeping principles have not changed.
- Counselors keep three major types of records: administrative records, recordings, and clinical case notes. It is important for you to understand what is involved in keeping each of these kinds of records and for you to follow sound procedures for securing them and protecting the privacy of your clients.
- When you begin working with clients, you will need to have clear and consistent policies and procedures for creating, maintaining, transferring, and destroying your records. Although your personal style and preferences may play some role, many of these policies and procedures will be dictated by your employer, laws, and accreditation standards.
- Many counselors are surprised to learn how many people can have access to their records. You should always make records with the assumption that they will be seen by others, including clients who have the right to access their records.

Three important federal laws that affect counseling records are the Health Insurance Portability and Accountability Act (HIPAA); the Family Educational Rights and Privacy Act (FERPA); and the Comprehensive Alcohol Abuse and Alcoholism Prevention, Treatment and Rehabilitation Act of 1972, a statute related to federally funded substance abuse programs. If you work in an educational institution or in an agency that receives federal funds to provide substance abuse treatment, you will need to understand the major provisions of these laws.

- HIPAA is a federal law that contains numerous detailed provisions regarding client privacy, informed consent, and transfer of records. Counselors in all settings must understand how this law affects their practices.
- FERPA is a federal law that protects the privacy of educational records. Among other things, FERPA requires educational institutions to obtain written permission of minor students or of adult students to transfer educational records to third parties and gives parents or students the right to inspect and challenge the contents of educational records.

The thought of receiving a subpoena might make you quite anxious. *Subpoenas* are official court documents that you cannot ignore.

- Unless you deal with subpoenas routinely in your work, you should consult your supervisor (if you are employed) or your practice attorney (if you are in private practice) whenever you receive a subpoena.
- Your attorney will be an invaluable resource in helping you respond appropriately to subpoenas and deal with legal proceedings such as interrogatories, depositions, and trials. Always seek the counsel of your supervisor or attorney and follow the advice you are given.